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Thermally regulated transdermal drug delivery system

Abstract

Described herein are drug and other active agent transdermal delivery systems and devices to deliver active agents to skin or mucosa of a subject, and methods of delivering such active agents. In particular, systems, methods, and devices are described that control the concentration and timing of active agent delivery. Such systems or devices may include a transdermal membrane and a temperature control element for heating and/or cooling a portion of the transdermal delivery system to provide pulsatile active agent delivery through the transdermal membrane.

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References Cited

U.S. PATENT DOCUMENTS

Patent No.	Issued Date	Patentee Name	U.S. Cl.	CPC
2183482	12/1938	Kurkjian	N/A	N/A
3279653	12/1965	Pfleger	N/A	N/A
3845217	12/1973	Ferno et al.	N/A	N/A
4313439	12/1981	Babb et al.	N/A	N/A
4321387	12/1981	Chavdarian et al.	N/A	N/A
4327725	12/1981	Cortese et al.	N/A	N/A
4332945	12/1981	Edwards	N/A	N/A
4379454	12/1982	Campbell et al.	N/A	N/A
4545990	12/1984	Le Foyer de Costil et al.	N/A	N/A
4579858	12/1985	Ferno et al.	N/A	N/A
4590278	12/1985	Edwards	N/A	N/A
4597961	12/1985	Etscorn	N/A	N/A
4708716	12/1986	Sibalis	N/A	N/A
4772263	12/1987	Dorman et al.	N/A	N/A
4806356	12/1988	Shaw	N/A	N/A
4853854	12/1988	Behar et al.	N/A	N/A
4885154	12/1988	Cormier et al.	N/A	N/A
4908213	12/1989	Govil et al.	N/A	N/A
4917676	12/1989	Heiber et al.	N/A	N/A
4917895	12/1989	Lee et al.	N/A	N/A
4920989	12/1989	Rose et al.	N/A	N/A
4952928	12/1989	Carroll et al.	N/A	N/A
4953572	12/1989	Rose et al.	N/A	N/A
4992445	12/1990	Lawter et al.	N/A	N/A
4994278	12/1990	Sablotsky	424/447	A61K 9/7038
5000956	12/1990	Amkraut et al.	N/A	N/A
5001139	12/1990	Lawter et al.	N/A	N/A
5013293	12/1990	Sibalis	N/A	N/A
5023252	12/1990	Hseih	N/A	N/A

5049387	12/1990	Amkraut	N/A	N/A
5069904	12/1990	Masterson	N/A	N/A
5073380	12/1990	Babu et al.	N/A	N/A
5097834	12/1991	Skrabal	N/A	N/A
5120545	12/1991	Ledger et al.	N/A	N/A
5130139	12/1991	Cormier et al.	N/A	N/A
5149538	12/1991	Granger et al.	N/A	N/A
5149719	12/1991	Ferber et al.	N/A	N/A
5212188	12/1992	Caldwell et al.	N/A	N/A
5221254	12/1992	Phipps	N/A	N/A
5227391	12/1992	Caldwell et al.	N/A	N/A
5232704	12/1992	Franz et al.	N/A	N/A
5232933	12/1992	Lippiello et al.	N/A	N/A
5236714	12/1992	Lee et al.	N/A	N/A
5242934	12/1992	Lippiello et al.	N/A	N/A
5242935	12/1992	Lippiello et al.	N/A	N/A
5242941	12/1992	Lewy et al.	N/A	N/A
5248690	12/1992	Caldwel et al.	N/A	N/A
5252604	12/1992	Nagy et al.	N/A	N/A
5262165	12/1992	Govil et al.	N/A	N/A
5273755	12/1992	Venkatraman et al.	N/A	N/A
5273756	12/1992	Fallon et al.	N/A	N/A
5304739	12/1993	Klug et al.	N/A	N/A
5310404	12/1993	Gyory et al.	N/A	N/A
5352456	12/1993	Fallon et al.	N/A	N/A
5364630	12/1993	Osborne et al.	N/A	N/A
5370635	12/1993	Strausak et al.	N/A	N/A
5388571	12/1994	Roberts et al.	N/A	N/A
5389679	12/1994	Alliger	N/A	N/A
5393526	12/1994	Castro	N/A	N/A
5405614	12/1994	D'Angelo et al.	N/A	N/A
5415629	12/1994	Henley	N/A	N/A
5445609	12/1994	Lattin et al.	N/A	N/A
5451407	12/1994	Cormier et al.	N/A	N/A
5464387	12/1994	Haak et al.	N/A	N/A
5472946	12/1994	Peck et al.	N/A	N/A
5501697	12/1995	Fisher	N/A	N/A
5505958	12/1995	Bello et al.	N/A	N/A
5512306	12/1995	Carlsson et al.	N/A	N/A
5516793	12/1995	Duffy	N/A	N/A
5525351	12/1995	Dam	N/A	N/A
5545407	12/1995	Hall et al.	N/A	N/A
5562607	12/1995	Gyory	N/A	N/A
5596994	12/1996	Bro	N/A	N/A
5601839	12/1996	Quan et al.	N/A	N/A
5616332	12/1996	Herstein	N/A	N/A
5618557	12/1996	Wille et al.	N/A	N/A
5653682	12/1996	Sibalis	N/A	N/A
5656255	12/1996	Jones	N/A	N/A
5662920	12/1996	Santus	N/A	N/A
5686100	12/1996	Wille et al.	N/A	N/A
5688232	12/1996	Flower	N/A	N/A

5697896	12/1996	McNichols et al.	N/A	N/A
5716987	12/1997	Wille	N/A	N/A
5722418	12/1997	Bro	N/A	N/A
5733259	12/1997	Valcke et al.	N/A	N/A
5785688	12/1997	Joshi et al.	N/A	N/A
5797867	12/1997	Guerrera et al.	N/A	N/A
5820875	12/1997	Fallon et al.	N/A	N/A
5833466	12/1997	Borg	N/A	N/A
5843979	12/1997	Wille et al.	N/A	N/A
5846559	12/1997	Hopp	N/A	N/A
5865786	12/1998	Sibalis et al.	N/A	N/A
5876368	12/1998	Flower	N/A	N/A
5879292	12/1998	Sternberg et al.	N/A	N/A
5879322	12/1998	Lattin et al.	N/A	N/A
5882676	12/1998	Lee et al.	N/A	N/A
5908301	12/1998	Lutz	N/A	N/A
5919156	12/1998	Stropkay et al.	N/A	N/A
5932240	12/1998	D'Angelo et al.	N/A	N/A
5945123	12/1998	Hermelin	N/A	N/A
5967789	12/1998	Segel et al.	N/A	N/A
5972389	12/1998	Shell et al.	N/A	N/A
5993435	12/1998	Haak et al.	N/A	N/A
5997501	12/1998	Gross et al.	N/A	N/A
6018679	12/1999	Dinh et al.	N/A	N/A
6019997	12/1999	Scholz et al.	N/A	N/A
6024981	12/1999	Khankari et al.	N/A	N/A
6034079	12/1999	Sanberg et al.	N/A	N/A
6059736	12/1999	Tapper	N/A	N/A
6059753	12/1999	Faust et al.	N/A	N/A
6068853	12/1999	Giannos et al.	N/A	N/A
6081734	12/1999	Batz	N/A	N/A
6090404	12/1999	Meconi et al.	N/A	N/A
6093419	12/1999	Rolf	N/A	N/A
6120803	12/1999	Wong et al.	N/A	N/A
6129702	12/1999	Woiias et al.	N/A	N/A
6162214	12/1999	Mueller et al.	N/A	N/A
6165155	12/1999	Jacobsen et al.	N/A	N/A
6211194	12/2000	Westman et al.	N/A	N/A
6211296	12/2000	Frate et al.	N/A	N/A
6221394	12/2000	Gilbert et al.	N/A	N/A
6238689	12/2000	Rhodes et al.	N/A	N/A
6274606	12/2000	Caldwell et al.	N/A	N/A
6310102	12/2000	Dull et al.	N/A	N/A
6365182	12/2001	Khankari et al.	N/A	N/A
6368625	12/2001	Siebert et al.	N/A	N/A
6374136	12/2001	Murdock	N/A	N/A
6416471	12/2001	Kumar et al.	N/A	N/A
6417359	12/2001	Crooks et al.	N/A	N/A
6423747	12/2001	Lanzendörfer et al.	N/A	N/A
6436078	12/2001	Svedman	N/A	N/A
6437004	12/2001	Perricone	N/A	N/A
6454708	12/2001	Ferguson et al.	N/A	N/A

6488959	12/2001	Stanley et al.	N/A	N/A
6492399	12/2001	Dull et al.	N/A	N/A
6539250	12/2002	Bettinger	N/A	N/A
6546281	12/2002	Zhang et al.	N/A	N/A
6567785	12/2002	Clendenon	N/A	N/A
6569449	12/2002	Stinchcomb et al.	N/A	N/A
6569866	12/2002	Simon	N/A	N/A
6576269	12/2002	Korneyev	N/A	N/A
6579865	12/2002	Mak et al.	N/A	N/A
6589229	12/2002	Connelly et al.	N/A	N/A
6595956	12/2002	Gross et al.	N/A	N/A
6638528	12/2002	Kanios	N/A	N/A
6638543	12/2002	Kang et al.	N/A	N/A
6660295	12/2002	Watanabe et al.	N/A	N/A
6689380	12/2003	Marchitto et al.	N/A	N/A
6723086	12/2003	Bassuk et al.	N/A	N/A
6723340	12/2003	Gusler et al.	N/A	N/A
6746688	12/2003	Kushnir et al.	N/A	N/A
6791003	12/2003	Choi et al.	N/A	N/A
6799576	12/2003	Farr	N/A	N/A
6849645	12/2004	Majeed et al.	N/A	N/A
6861066	12/2004	Van de Casteele	N/A	N/A
6867342	12/2004	Johnston et al.	N/A	N/A
6887202	12/2004	Currie et al.	N/A	N/A
6893655	12/2004	Flanigan et al.	N/A	N/A
6900202	12/2004	Imoto et al.	N/A	N/A
6911475	12/2004	Villafane et al.	N/A	N/A
6998176	12/2005	Morita et al.	N/A	N/A
7011843	12/2005	Becher et al.	N/A	N/A
7011849	12/2005	Storm et al.	N/A	N/A
7019622	12/2005	Orr et al.	N/A	N/A
7064143	12/2005	Gurley et al.	N/A	N/A
7182955	12/2006	Hart et al.	N/A	N/A
7196619	12/2006	Perlman et al.	N/A	N/A
7229641	12/2006	Cherukuri	N/A	N/A
7282217	12/2006	Grimshaw et al.	N/A	N/A
7332182	12/2007	Sackler	N/A	N/A
7376700	12/2007	Clark et al.	N/A	N/A
7384651	12/2007	Hille et al.	N/A	N/A
7384653	12/2007	Wright et al.	N/A	N/A
7579019	12/2008	Tapolsky et al.	N/A	N/A
7598275	12/2008	Cooke et al.	N/A	N/A
7718677	12/2009	Quik et al.	N/A	N/A
7780981	12/2009	DiPierro et al.	N/A	N/A
7931563	12/2010	Shaw et al.	N/A	N/A
7988660	12/2010	Byland et al.	N/A	N/A
8003080	12/2010	Rabinowitz et al.	N/A	N/A
8021334	12/2010	Shekalim	N/A	N/A
8137314	12/2011	Mounce et al.	N/A	N/A
8140143	12/2011	Picard et al.	N/A	N/A
8192756	12/2011	Berner et al.	N/A	N/A
8246581	12/2011	Adams et al.	N/A	N/A

8252321	12/2011	DiPierro et al.	N/A	N/A
8262394	12/2011	Walker et al.	N/A	N/A
8268475	12/2011	Tucholski	N/A	N/A
8285328	12/2011	Caffey et al.	N/A	N/A
8303500	12/2011	Raheman	N/A	N/A
8309568	12/2011	Stinchcomb et al.	N/A	N/A
8372040	12/2012	Huang et al.	N/A	N/A
8414532	12/2012	Brandt et al.	N/A	N/A
8440220	12/2012	Gale et al.	N/A	N/A
8440221	12/2012	Zumbrunn et al.	N/A	N/A
8441411	12/2012	Tucholski et al.	N/A	N/A
8445010	12/2012	Anderson et al.	N/A	N/A
8449471	12/2012	Tran	N/A	N/A
8517988	12/2012	Smith	N/A	N/A
8545445	12/2012	Kamen et al.	N/A	N/A
8574188	12/2012	Potter et al.	N/A	N/A
8586079	12/2012	Hansted et al.	N/A	N/A
8589174	12/2012	Nelson et al.	N/A	N/A
8614278	12/2012	Loubert et al.	N/A	N/A
8632497	12/2013	Yodfat et al.	N/A	N/A
8666781	12/2013	Hanina et al.	N/A	N/A
8673346	12/2013	Zumbrunn et al.	N/A	N/A
8684922	12/2013	Tran	N/A	N/A
8688189	12/2013	Shennib	N/A	N/A
8690827	12/2013	Edwards et al.	N/A	N/A
8690865	12/2013	Prausnitz et al.	N/A	N/A
8696637	12/2013	Ross	N/A	N/A
8703175	12/2013	Kanios et al.	N/A	N/A
8703177	12/2013	Finn et al.	N/A	N/A
8722233	12/2013	Tucholski	N/A	N/A
8727745	12/2013	Rush et al.	N/A	N/A
8741336	12/2013	DiPierro et al.	N/A	N/A
8747348	12/2013	Yodfat et al.	N/A	N/A
8753315	12/2013	Alferness et al.	N/A	N/A
8773257	12/2013	Yodfat et al.	N/A	N/A
8814822	12/2013	Yodfat et al.	N/A	N/A
8862223	12/2013	Yanaki	N/A	N/A
8864727	12/2013	Lee	N/A	N/A
8865207	12/2013	Kanios et al.	N/A	N/A
8872663	12/2013	Forster	N/A	N/A
8876802	12/2013	Grigorov	N/A	N/A
8956644	12/2014	Yum et al.	N/A	N/A
8962014	12/2014	Prinz et al.	N/A	N/A
8986253	12/2014	DiPerna	N/A	N/A
8999356	12/2014	Ramirez et al.	N/A	N/A
8999372	12/2014	Davidson et al.	N/A	N/A
9023392	12/2014	Koo et al.	N/A	N/A
9044582	12/2014	Chang et al.	N/A	N/A
9050348	12/2014	Kydonieus et al.	N/A	N/A
9078833	12/2014	Audett	N/A	N/A
9111085	12/2014	Darmour et al.	N/A	N/A
9114240	12/2014	Horstmann et al.	N/A	N/A

9155712	12/2014	Kanios et al.	N/A	N/A
9233203	12/2015	Moberg et al.	N/A	N/A
9238001	12/2015	Weyer et al.	N/A	N/A
9238108	12/2015	Edwards et al.	N/A	N/A
9248104	12/2015	Valia et al.	N/A	N/A
9289397	12/2015	Wright	N/A	N/A
9308202	12/2015	Hille et al.	N/A	N/A
9314527	12/2015	Cottrell et al.	N/A	N/A
9373269	12/2015	Bergman et al.	N/A	N/A
9380698	12/2015	Li et al.	N/A	N/A
RE46217	12/2015	Huang et al.	N/A	N/A
9513666	12/2015	Li et al.	N/A	N/A
9549903	12/2016	Hille et al.	N/A	N/A
9555226	12/2016	Zumbrunn et al.	N/A	N/A
9555227	12/2016	Dipierro	N/A	N/A
9555277	12/2016	Yeh	N/A	N/A
9623017	12/2016	Barbier et al.	N/A	N/A
9636457	12/2016	Newberry et al.	N/A	N/A
9655843	12/2016	Finn et al.	N/A	N/A
9656441	12/2016	LeDonne et al.	N/A	N/A
9669199	12/2016	DiPierro et al.	N/A	N/A
9687186	12/2016	Goldstein et al.	N/A	N/A
9693689	12/2016	Gannon et al.	N/A	N/A
9700552	12/2016	Weimann	N/A	N/A
9717698	12/2016	Horstmann et al.	N/A	N/A
9735893	12/2016	Aleksov et al.	N/A	N/A
9782082	12/2016	Gannon et al.	N/A	N/A
9795681	12/2016	Abreu	N/A	N/A
9867539	12/2017	Heikenfeld et al.	N/A	N/A
9895320	12/2017	Ogino et al.	N/A	N/A
9949935	12/2017	Murata	N/A	N/A
9974492	12/2017	Dicks et al.	N/A	N/A
9993203	12/2017	Mei et al.	N/A	N/A
10004447	12/2017	Shen et al.	N/A	N/A
10034841	12/2017	Müller et al.	N/A	N/A
10105487	12/2017	DiPierro et al.	N/A	N/A
10143687	12/2017	Azhir	N/A	N/A
10213586	12/2018	Netzel et al.	N/A	N/A
10232156	12/2018	Netzel et al.	N/A	N/A
10258738	12/2018	Dipierro et al.	N/A	N/A
10258778	12/2018	DiPierro et al.	N/A	N/A
10679516	12/2019	Darmour et al.	N/A	N/A
10716764	12/2019	Zumbrunn et al.	N/A	N/A
2001/0022978	12/2000	Lacharriere et al.	N/A	N/A
2001/0026788	12/2000	Piskorz	N/A	N/A
2002/0002189	12/2001	Smith et al.	N/A	N/A
2002/0034535	12/2001	Kleiner et al.	N/A	N/A
2002/0106329	12/2001	Leslie	N/A	N/A
2002/0127256	12/2001	Murad	N/A	N/A
2002/0165170	12/2001	Wilson et al.	N/A	N/A
2002/0169439	12/2001	Flaherty	N/A	N/A
2002/0182238	12/2001	Creton	N/A	N/A

2003/0004187	12/2002	Bedard et al.	N/A	N/A
2003/0065294	12/2002	Pickup et al.	N/A	N/A
2003/0065924	12/2002	Wuidart et al.	N/A	N/A
2003/0083645	12/2002	Angel et al.	N/A	N/A
2003/0087937	12/2002	Lindberg	N/A	N/A
2003/0119879	12/2002	Landh et al.	N/A	N/A
2003/0138464	12/2002	Zhang	604/20	A61K 9/0024
2003/0159702	12/2002	Lindell et al.	N/A	N/A
2004/0019321	12/2003	Sage et al.	N/A	N/A
2004/0034068	12/2003	Warchol et al.	N/A	N/A
2004/0037879	12/2003	Adusumilli et al.	N/A	N/A
2004/0052843	12/2003	Lerner et al.	N/A	N/A
2004/0062802	12/2003	Hermelin	N/A	N/A
2004/0138074	12/2003	Ahmad et al.	N/A	N/A
2004/0166159	12/2003	Han et al.	N/A	N/A
2004/0191322	12/2003	Hansson	N/A	N/A
2004/0194793	12/2003	Lindell et al.	N/A	N/A
2004/0219192	12/2003	Horstmann et al.	N/A	N/A
2004/0229908	12/2003	Nelson	N/A	N/A
2004/0241218	12/2003	Tavares et al.	N/A	N/A
2004/0253249	12/2003	Rudnic et al.	N/A	N/A
2004/0259816	12/2003	Pandol et al.	N/A	N/A
2005/0002806	12/2004	Fuechslin et al.	N/A	N/A
2005/0014779	12/2004	Papke	N/A	N/A
2005/0021092	12/2004	Yun et al.	N/A	N/A
2005/0034842	12/2004	Huber et al.	N/A	N/A
2005/0048020	12/2004	Wille	N/A	N/A
2005/0053665	12/2004	Ek et al.	N/A	N/A
2005/0113452	12/2004	Flashner Barak et al.	N/A	N/A
2005/0141346	12/2004	Rawls et al.	N/A	N/A
2005/0151110	12/2004	Minor et al.	N/A	N/A
2005/0159419	12/2004	Stephenson et al.	N/A	N/A
2005/0197625	12/2004	Haueter et al.	N/A	N/A
2005/0266032	12/2004	Srinivasan et al.	N/A	N/A
2005/0276852	12/2004	Davis et al.	N/A	N/A
2006/0024358	12/2005	Santini et al.	N/A	N/A
2006/0036209	12/2005	Subramony et al.	N/A	N/A
2006/0057202	12/2005	Antarkar et al.	N/A	N/A
2006/0122577	12/2005	Poulsen et al.	N/A	N/A
2006/0135911	12/2005	Mittur	604/113	A61F 7/007
2006/0167039	12/2005	Nguyen et al.	N/A	N/A
2006/0173439	12/2005	Thorne et al.	N/A	N/A
2006/0184093	12/2005	Phipps et al.	N/A	N/A
2006/0188859	12/2005	Yakobi	N/A	N/A
2006/0204578	12/2005	Vergez et al.	N/A	N/A
2006/0206054	12/2005	Shekalim	N/A	N/A
2007/0026054	12/2006	Theobald et al.	N/A	N/A
2007/0042026	12/2006	Wille	N/A	N/A
2007/0065365	12/2006	Kugelmann et al.	N/A	N/A
2007/0086275	12/2006	Robinson et al.	N/A	N/A
2007/0088338	12/2006	Ehwald et al.	N/A	N/A

2007/0104787	12/2006	Posey Dowty et al.	N/A	N/A
2007/0149952	12/2006	Bland et al.	N/A	N/A
2007/0154336	12/2006	Miyazaki et al.	N/A	N/A
2007/0168501	12/2006	Cobb et al.	N/A	N/A
2007/0179172	12/2006	Becker et al.	N/A	N/A
2007/0191815	12/2006	DiPierro	N/A	N/A
2007/0250018	12/2006	Adachi et al.	N/A	N/A
2007/0255195	12/2006	Adachi	N/A	N/A
2007/0256684	12/2006	Kelliher et al.	N/A	N/A
2007/0260491	12/2006	Palmer et al.	N/A	N/A
2007/0279217	12/2006	Venkatraman et al.	N/A	N/A
2007/0299401	12/2006	Alferness et al.	N/A	N/A
2008/0008747	12/2007	Royds	N/A	N/A
2008/0015494	12/2007	Santini et al.	N/A	N/A
2008/0045879	12/2007	Prausnitz	606/41	A61B 18/082
2008/0131494	12/2007	Reed et al.	N/A	N/A
2008/0138294	12/2007	Gonda	N/A	N/A
2008/0138398	12/2007	Gonda	N/A	N/A
2008/0138399	12/2007	Gonda	N/A	N/A
2008/0138423	12/2007	Gonda	N/A	N/A
2008/0139907	12/2007	Rao et al.	N/A	N/A
2008/0152592	12/2007	Rebec	N/A	N/A
2008/0195946	12/2007	Peri-Glass	N/A	N/A
2008/0201174	12/2007	Ramasubramanian et al.	N/A	N/A
2008/0233178	12/2007	Reidenberg et al.	N/A	N/A
2008/0274168	12/2007	Baker et al.	N/A	N/A
2008/0319272	12/2007	Patangay et al.	N/A	N/A
2009/0005009	12/2008	Marsili	N/A	N/A
2009/0010998	12/2008	Marchitto et al.	N/A	N/A
2009/0024004	12/2008	Yang	N/A	N/A
2009/0028824	12/2008	Chiang et al.	N/A	N/A
2009/0062754	12/2008	Tang	N/A	N/A
2009/0118710	12/2008	Kortzeborn	N/A	N/A
2009/0169631	12/2008	Zamlot et al.	N/A	N/A
2009/0246265	12/2008	Stinchcomb et al.	N/A	N/A
2009/0247985	12/2008	Melsheimer et al.	N/A	N/A
2009/0259176	12/2008	Yairi	N/A	N/A
2009/0299156	12/2008	Simpson et al.	N/A	N/A
2010/0003653	12/2009	Brown	N/A	N/A
2010/0010443	12/2009	Morgan et al.	N/A	N/A
2010/0068250	12/2009	Anderson et al.	N/A	N/A
2010/0114008	12/2009	Marchitto et al.	N/A	N/A
2010/0130932	12/2009	Yodfat et al.	N/A	N/A
2010/0145303	12/2009	Yodfat et al.	N/A	N/A
2010/0179473	12/2009	Genosar	N/A	N/A
2010/0196463	12/2009	Quik et al.	N/A	N/A
2010/0198187	12/2009	Yodfat et al.	N/A	N/A
2010/0211005	12/2009	Edwards et al.	N/A	N/A
2010/0248198	12/2009	Seidman et al.	N/A	N/A
2010/0273738	12/2009	Valcke et al.	N/A	N/A
2010/0280432	12/2009	DiPierro et al.	N/A	N/A

2011/0004072	12/2010	Fletcher et al.	N/A	N/A
2011/0053129	12/2010	Basson et al.	N/A	N/A
2011/0054285	12/2010	Searle et al.	N/A	N/A
2011/0109439	12/2010	Borlenghi	N/A	N/A
2011/0137255	12/2010	Nielsen et al.	N/A	N/A
2011/0152635	12/2010	Morris et al.	N/A	N/A
2011/0153360	12/2010	Hanina et al.	N/A	N/A
2011/0160640	12/2010	Yanaki	N/A	N/A
2011/0160655	12/2010	Hanson et al.	N/A	N/A
2011/0212027	12/2010	Hoare	424/9.1	A61K 9/0024
2011/0241446	12/2010	Tucholski	N/A	N/A
2011/0245633	12/2010	Goldberg et al.	N/A	N/A
2011/0245783	12/2010	Stinchcomb et al.	N/A	N/A
2011/0250576	12/2010	Hester	N/A	N/A
2011/0256517	12/2010	Swanson	N/A	N/A
2011/0264028	12/2010	Ramdas et al.	N/A	N/A
2011/0268809	12/2010	Brinkley et al.	N/A	N/A
2011/0275987	12/2010	Caffey et al.	N/A	N/A
2012/0046644	12/2011	Ziaie et al.	N/A	N/A
2012/0078216	12/2011	Smith et al.	N/A	N/A
2012/0123387	12/2011	Gonzalez et al.	N/A	N/A
2012/0156138	12/2011	Smith	N/A	N/A
2012/0171277	12/2011	Royds	N/A	N/A
2012/0178065	12/2011	Naghavi et al.	N/A	N/A
2012/0191043	12/2011	Yodfat et al.	N/A	N/A
2012/0203573	12/2011	Mayer et al.	N/A	N/A
2012/0209223	12/2011	Salman et al.	N/A	N/A
2012/0221251	12/2011	Rosenberg et al.	N/A	N/A
2012/0244503	12/2011	Neveldine	N/A	N/A
2012/0302844	12/2011	Schnidrig et al.	N/A	N/A
2012/0316896	12/2011	Rahman et al.	N/A	N/A
2012/0329017	12/2011	Pham	N/A	N/A
2013/0017259	12/2012	Azhir	N/A	N/A
2013/0041258	12/2012	Patrick et al.	N/A	N/A
2013/0096495	12/2012	Holmqvist et al.	N/A	N/A
2013/0123719	12/2012	Mao et al.	N/A	N/A
2013/0158430	12/2012	Aceti et al.	N/A	N/A
2013/0178826	12/2012	Li	N/A	N/A
2013/0190683	12/2012	Hanson et al.	N/A	N/A
2013/0216989	12/2012	Cuthbert	N/A	N/A
2013/0253430	12/2012	Kouyoumjian et al.	N/A	N/A
2013/0302398	12/2012	Ambati et al.	N/A	N/A
2013/0311917	12/2012	Bar-or et al.	N/A	N/A
2013/0317384	12/2012	Le	N/A	N/A
2013/0328572	12/2012	Wang et al.	N/A	N/A
2013/0345633	12/2012	Chong	N/A	N/A
2014/0039396	12/2013	Geipel et al.	N/A	N/A
2014/0046288	12/2013	Geipel et al.	N/A	N/A
2014/0073883	12/2013	Rao et al.	N/A	N/A
2014/0088554	12/2013	Li et al.	N/A	N/A
2014/0099614	12/2013	Hu et al.	N/A	N/A

2014/0100241	12/2013	Slater et al.	N/A	N/A
2014/0163521	12/2013	O'Conner	N/A	N/A
2014/0200525	12/2013	DiPierro	N/A	N/A
2014/0206327	12/2013	Ziemianska et al.	N/A	N/A
2014/0207047	12/2013	DiPierro et al.	N/A	N/A
2014/0228736	12/2013	Eppstein et al.	N/A	N/A
2014/0237028	12/2013	Messenger et al.	N/A	N/A
2014/0240124	12/2013	Bychkov	N/A	N/A
2014/0266584	12/2013	Ingle et al.	N/A	N/A
2014/0272844	12/2013	Hendriks et al.	N/A	N/A
2014/0272845	12/2013	Hendriks et al.	N/A	N/A
2014/0272846	12/2013	Richling	N/A	N/A
2014/0275135	12/2013	Genov et al.	N/A	N/A
2014/0275932	12/2013	Zadig	N/A	N/A
2014/0276127	12/2013	Ferdosi et al.	N/A	N/A
2014/0279740	12/2013	Wernevi et al.	N/A	N/A
2014/0302121	12/2013	Bevier	N/A	N/A
2014/0303574	12/2013	Knutson	N/A	N/A
2014/0365408	12/2013	Snyder et al.	N/A	N/A
2014/0378943	12/2013	Geipel	N/A	N/A
2015/0057616	12/2014	Shergold et al.	N/A	N/A
2015/0209783	12/2014	Ingber et al.	N/A	N/A
2015/0273148	12/2014	Sexton et al.	N/A	N/A
2015/0310760	12/2014	Knotts et al.	N/A	N/A
2015/0322939	12/2014	Katase	N/A	N/A
2015/0342900	12/2014	Putnins	N/A	N/A
2016/0030412	12/2015	Azhir	N/A	N/A
2016/0058939	12/2015	Brewer et al.	N/A	N/A
2016/0220553	12/2015	Azhir	N/A	N/A
2016/0227361	12/2015	Booth et al.	N/A	N/A
2016/0228383	12/2015	Zhang et al.	N/A	N/A
2016/0235732	12/2015	Quik et al.	N/A	N/A
2016/0235916	12/2015	Edwards et al.	N/A	N/A
2016/0263312	12/2015	Junod et al.	N/A	N/A
2016/0310664	12/2015	McKenzie et al.	N/A	N/A
2016/0317057	12/2015	Li et al.	N/A	N/A
2016/0317738	12/2015	Cross et al.	N/A	N/A
2016/0339174	12/2015	Shapley et al.	N/A	N/A
2016/0346456	12/2015	Cefai et al.	N/A	N/A
2016/0346462	12/2015	Adams et al.	N/A	N/A
2017/0007550	12/2016	Enscore et al.	N/A	N/A
2017/0079932	12/2016	Emgenbroich et al.	N/A	N/A
2017/0100573	12/2016	DiPierro	N/A	N/A
2017/0189348	12/2016	Lee et al.	N/A	N/A
2017/0189534	12/2016	Lee et al.	N/A	N/A
2017/0207825	12/2016	Belogolovy	N/A	N/A
2017/0209429	12/2016	Stinchcomb et al.	N/A	N/A
2017/0232192	12/2016	Sasaki	N/A	N/A
2017/0249433	12/2016	Hagen et al.	N/A	N/A
2017/0296107	12/2016	Reid et al.	N/A	N/A
2017/0296317	12/2016	Gordon	N/A	N/A
2017/0351840	12/2016	Goguen	N/A	N/A

2018/0014783	12/2017	Shi et al.	N/A	N/A
2018/0028069	12/2017	Shi et al.	N/A	N/A
2018/0028070	12/2017	Shi	N/A	N/A
2018/0028071	12/2017	Shi	N/A	N/A
2018/0028072	12/2017	Shi	N/A	N/A
2018/0110768	12/2017	Quik et al.	N/A	N/A
2018/0110975	12/2017	Ivanoff et al.	N/A	N/A
2018/0165566	12/2017	Rogers et al.	N/A	N/A
2018/0168504	12/2017	Ding et al.	N/A	N/A
2018/0197637	12/2017	Chowdhury	N/A	N/A
2018/0374381	12/2017	Darmour et al.	N/A	N/A
2019/0000828	12/2018	Azhir	N/A	N/A
2019/0009019	12/2018	Shor	N/A	A61M 5/1782
2019/0054078	12/2018	Azhir et al.	N/A	N/A
2019/0054235	12/2018	DiPierro et al.	N/A	N/A
2019/0231707	12/2018	Stiles et al.	N/A	N/A
2019/0275308	12/2018	Netzel et al.	N/A	N/A
2019/0336738	12/2018	Johnson et al.	N/A	N/A
2019/0374482	12/2018	Schaller et al.	N/A	N/A
2020/0030590	12/2019	Buchman et al.	N/A	N/A
2020/0330369	12/2019	DiPierro	N/A	N/A
2020/0368175	12/2019	Arora et al.	N/A	N/A
2021/0169822	12/2020	Zumbrunn et al.	N/A	N/A
2021/0196935	12/2020	Tong et al.	N/A	N/A
2024/0408362	12/2023	Johnston et al.	N/A	N/A
2024/0416095	12/2023	Tong et al.	N/A	N/A
2024/0424270	12/2023	Netzel et al.	N/A	N/A

FOREIGN PATENT DOCUMENTS

Patent No.	Application Date	Country	CPC
662877	12/1994	AU	N/A
899037	12/1983	BE	N/A
2142871	12/1993	CA	N/A
1704056	12/2004	CN	N/A
19958554	12/2000	DE	N/A
10105759	12/2000	DE	N/A
10103158	12/2001	DE	N/A
311313	12/1988	EP	N/A
0314528	12/1991	EP	N/A
0354554	12/1993	EP	N/A
0726005	12/1995	EP	N/A
857725	12/1997	EP	N/A
870768	12/1997	EP	N/A
955301	12/1998	EP	N/A
0612525	12/2000	EP	N/A
1815784	12/2006	EP	N/A
1977746	12/2013	EP	N/A
1662989	12/2013	EP	N/A
3016586	12/2015	EP	N/A
1528391	12/1977	GB	N/A

2030862	12/1979	GB	N/A
2142822	12/1984	GB	N/A
2230439	12/1989	GB	N/A
02202813	12/1989	JP	N/A
H09504974	12/1996	JP	N/A
09512006	12/1996	JP	N/A
2000515394	12/1999	JP	N/A
2001505491	12/2000	JP	N/A
2002092180	12/2001	JP	N/A
2003506477	12/2002	JP	N/A
2005521526	12/2004	JP	N/A
2005525147	12/2004	JP	N/A
2007509661	12/2006	JP	N/A
2008523918	12/2007	JP	N/A
2009544338	12/2008	JP	N/A
2010/507457	12/2009	JP	N/A
2010518914	12/2009	JP	N/A
2010279808	12/2009	JP	N/A
2011036491	12/2010	JP	N/A
2013524951	12/2012	JP	N/A
2015070868	12/2014	JP	N/A
2016202904	12/2015	JP	N/A
20180011653	12/2017	KR	A61M 37/0015
WO86/07269	12/1985	WO	N/A
WO88/003803	12/1987	WO	N/A
WO91/14441	12/1990	WO	N/A
WO92/021339	12/1991	WO	N/A
WO94/008992	12/1993	WO	N/A
WO94/010987	12/1993	WO	N/A
WO95/06497	12/1994	WO	N/A
WO96/015123	12/1995	WO	N/A
WO96/040682	12/1995	WO	N/A
WO97/011072	12/1996	WO	N/A
WO97/011073	12/1996	WO	N/A
WO97/11741	12/1996	WO	N/A
WO97/18782	12/1996	WO	N/A
WO97/019059	12/1996	WO	N/A
WO97/028801	12/1996	WO	N/A
WO97/034605	12/1996	WO	N/A
WO97/042941	12/1996	WO	N/A
WO97/046554	12/1996	WO	N/A
WO98/042713	12/1997	WO	N/A
WO98/46093	12/1997	WO	N/A
WO98/054152	12/1997	WO	N/A
WO98/054181	12/1997	WO	N/A
WO98/054182	12/1997	WO	N/A
WO98/054189	12/1997	WO	N/A
WO98/55107	12/1997	WO	N/A
WO99/002517	12/1998	WO	N/A
WO99/003859	12/1998	WO	N/A
WO99/021834	12/1998	WO	N/A

WO99/024422	12/1998	WO	N/A
WO99/066916	12/1998	WO	N/A
WO00/010997	12/1999	WO	N/A
WO00/032600	12/1999	WO	N/A
WO00/034279	12/1999	WO	N/A
WO00/034284	12/1999	WO	N/A
WO00/035279	12/1999	WO	N/A
WO00/035456	12/1999	WO	N/A
WO00/044755	12/1999	WO	N/A
WO00/064885	12/1999	WO	N/A
WO00/066596	12/1999	WO	N/A
WO00/74763	12/1999	WO	N/A
WO00/74933	12/1999	WO	N/A
WO01/005459	12/2000	WO	N/A
WO01/037814	12/2000	WO	N/A
WO02/076211	12/2001	WO	N/A
WO03/022349	12/2002	WO	N/A
WO03/026655	12/2002	WO	N/A
WO03/055486	12/2002	WO	N/A
WO03/061656	12/2002	WO	N/A
WO03/070191	12/2002	WO	N/A
WO03/097146	12/2002	WO	N/A
WO2004/024124	12/2003	WO	N/A
WO2004/073429	12/2003	WO	N/A
WO2005/023227	12/2004	WO	N/A
WO2005/079161	12/2004	WO	N/A
WO2006/069097	12/2005	WO	N/A
WO2007/013975	12/2006	WO	N/A
WO2007/041544	12/2006	WO	N/A
WO2007/104574	12/2006	WO	N/A
WO2007/104575	12/2006	WO	N/A
WO2007/133141	12/2006	WO	N/A
WO2008/024408	12/2007	WO	N/A
WO2008/054788	12/2007	WO	N/A
WO2008/069921	12/2007	WO	N/A
WO2008/069970	12/2007	WO	N/A
WO2008/069972	12/2007	WO	N/A
WO2008/122049	12/2007	WO	N/A
WO2008/135283	12/2007	WO	N/A
WO2009/136304	12/2008	WO	N/A
WO2011/088072	12/2010	WO	N/A
WO2012/012846	12/2011	WO	N/A
WO2012/101060	12/2011	WO	N/A
WO2013/093666	12/2012	WO	N/A
WO2013/168068	12/2012	WO	N/A
WO2014/001877	12/2013	WO	N/A
WO2014/043502	12/2013	WO	N/A
WO2016/081616	12/2015	WO	N/A
WO2016/132368	12/2015	WO	N/A
WO2016/161416	12/2015	WO	N/A
WO2017/053938	12/2016	WO	N/A
WO2017/125455	12/2016	WO	N/A

WO2018/026759	12/2017	WO	N/A
WO2018/129363	12/2017	WO	N/A
WO2018/133786	12/2017	WO	N/A
WO2019/090125	12/2018	WO	N/A
WO2019/232077	12/2018	WO	N/A

OTHER PUBLICATIONS

Abood et al.; Structure-activity studies of carbamate and other esters: agonists and antagonists to nicotine; Pharmacology Biochemistry and Behavior; 30(2); pp. 403-408; Jun. 1988. cited by applicant

Ahlskog et al.; Frequency of levodopa-related dyskinesias and motor fluctuations as estimated from the cumulative literature; Movement Disorders; 16(3); pp. 448-458; May 1, 2001. cited by applicant

Angulo et al.; Oral nicotine in treatment of primary sclerosing cholangitis: a pilot study; Digestive diseases and sciences; 44(3); pp. 602-607; Mar. 1, 1999. cited by applicant

Azhir, Arasteh; U.S. Appl. No. 62/320,871 entitled "Compositions and methods for treatment related to fall and fall frequency in neurodegenerative diseases", filed Apr. 11, 2016. cited by applicant

Baldessarini et al.; Preclinical studies of the pharmacology of aporphines; In: Gessa GL, Corsini GU, eds.; Apomorphine and other dopaminomi-'metics vol. 1, Basic pharmacology; New York: Raven Press; pp. 219-228; (year of pub. sufficiently earlier than effective US filing date and any foreign priority date) 1981. cited by applicant

Balfour et al.; Pharmacology of nicotine and its therapeutic use in smoking cessation and neurodegenerative disorders; Pharmacology and Therapeutics; 72(1); pp. 51-81; Jan. 1, 1996. cited by applicant

Benowitz et al.; Sources of variability in nicotine and cotinine levels with use of nicotine nasal spray, transdermal nicotine, and cigarette smoking; British Journal of Clinical Pharmacology; 43(3); pp. 259-267; Mar. 1, 1997. cited by applicant

Benowitz et al.; Stable isotope studies of nicotine kinetics and bioavailability; Clin Pharm and Ther; 49(3); pp. 270-277; Mar. 1991. cited by applicant

Bordia et al.; Continuous and intermittent nicotine treatment reduces L-3 4-dihydroxyphenylalanine (L-DOPA)-induced dyskinesias in rat model of Parkinson's diseases; Journal of Pharmacology and Experimental Therapeutics; 327(1); pp. 239-247; Oct. 1, 2008. cited by applicant

Bordia et al.; Partial recovery of striatal nicotinic receptors in l-methyl-4-phenyl-1,2,3,6-tetrahydropyridine (MPTP)-lesioned monkeys with chronic oral nicotinic; The Journal of Pharmacology and Experimental Therapeutics; 319(1); pp. 285-292; Oct. 1, 2006. cited by applicant

Bove et al.; Toxin-induced models of Parkinson's disease; NeuroRx; 2(3); pp. 484-494; Jul. 31, 2005. cited by applicant

Bricker et al.; Randomized controlled pilot trial of a smartphone app for smoking cessation using acceptance and commitment therapy; Drug and Alcohol Dependence; 143; pp. 87-94; Oct. 1, 2014 (Author Manuscript). cited by applicant

Brotchie et al.; Levodopa-induced dyskinesia in Parkinson's disease; Journal of Neural Transmission; 112(3); pp. 359-391; Mar. 1, 2005. cited by applicant

Bruguerolle; Chronopharmacokinetics; Clin Pharmacokinet; 35(2); pp. 83-94; Aug. 1998. cited by applicant

Calabresi et al.; Levodopa-induced dyskinesias inpatients with parkinson's disease: filling the bench-to-bedside gap; The Lancet Neurology; 9(11); pp. 1106-1117; Nov. 1, 2010. cited by applicant

Carta et al.; Role of striatal L-DOPA in the production of dyskinesia in 6-hydroxydopamine lesioned rats; Journal of Neurochemistry; 96(6); pp. 1718-1727; Mar. 2006. cited by applicant

Chen et al.; Enhanced striatal opioid receptor-mediated G-protein activation in L-DOPA-treated dyskinetic monkeys; Neuroscience; 132(2); pp. 409-420; Dec. 31, 2005. cited by applicant

Damaj et al.; Antinociceptive responses to nicotinic acetylcholine receptor ligands after systemic and intrathecal administration in mice; Journal of Pharmacology and Experimental Therapeutics; 284(3); pp. 1058-1065; Mar. 1, 1998. cited by applicant

Davie; A review of Parkinson's disease. British Medical Bulletin 2008 86(1): 109-127; Apr. 8, 2008. cited by applicant

De La Fuente et al.; The placebo effect in Parkinson's disease; Trends in Neuroscience; 25(6); pp. 302-306;

Jun. 1, 2002. cited by applicant

Di Monte et al.; Relationship among nigrostriatal denervation, parkinsonism, and dyskinesias in the MPTP primate model; *Movement Disorders*; 15(3); pp. 459-466; May 1, 2000. cited by applicant

Dockser-Marcus, A.; New research shows drugs work best at certain times; *The Wall Street Journal*; 6 pgs.; May 27, 2003; (<http://www.wsj.com/articles/SB105397312486508700>). cited by applicant

Domino et al.; Nicotine alone and in combination with L-DOPA methyl ester or the D(2) agonist N-0923 in MPTP-induced chronic hemiparkinsonian monkeys; *Exp Neurol*; 158(2); pp. 414-421; Aug. 1999. cited by applicant

Dutil; Benzoyl Peroxide: Enhancing antibiotic efficacy in acne management; *Skin Therapy Letter*; 15(1); pp. 5-7; Nov./Dec. 2010. cited by applicant

Ebersbach et al.; Worsening of motor performance in patients with Parkinson's disease following transdermal nicotine administration; *Movement Disorders*; 14(6); pp. 1011-1013; Nov. 1, 1999. cited by applicant

Ethicon Endo-Surgery, Inc.; Sedasys® Computer-assisted personalized sedation system essential product information; retrieved May 12, 2015 from the internet (<http://www.sedasy.com/explore-the-system/essential-product-information>); 2 pgs. cited by applicant

Fagerstrom et al.; Nicotine may relieve symptoms of Parkinson's disease; *Psychopharmacology*; 116(1); pp. 117-119; Sep. 16, 1994. cited by applicant

Food and Drug Administration; Guidance for Industry—Dissolution Testing of Immediate Release Solid Oral Dosage Forms; 17 pages; retrieved from the internet (<https://www.fda.gov/downloads/drugs/guidances/ucm070237.pdf>); Aug. 1997. cited by applicant

Gatto et al.; TC-1734: An orally active neuronal nicotinic acetylcholine receptor modulator with antidepressant, neuroprotective and long-lasting cognitive effects; *CNS Drug Reviews*; 10(2); pp. 147-166; Jun. 1, 2004. cited by applicant

Gennaro (Editor); Remington: The Science and Practice of Pharmacy; 19th Ed.; Mack Publishing Co.; Easton, PA; p. 1582-1584; Jun. 1995. cited by applicant

Giannos; Chapter 20: Pulsatile fSmartf Drug Delivery, in *Skin Delivery Systems: Transdermals, Dermatologicals, and Cosmetic Actives*; (ed.) Wille, Jr; Blackwell Pub.; Oxford, UK; pp. 327-357; Jun. 2006. cited by applicant

Gora; Nicotine transdermal systems; *The Annals of Pharmacotherapy*; 27(6); pp. 742-750; Jun. 1993. cited by applicant

Gotti et al.; Brain nicotinic acetylcholine receptors: native subtypes and their relevance; *Trends in Pharmacological Sciences*; 27(9); pp. 482-491; Sep. 30, 2006. cited by applicant

Green et al.; An oral formulation of nicotine for release and absorption in the colon: its development and pharmacokinetics. *British Journal of Clinical Pharmacology*; 48(4); pp. 485-493; Oct. 1999. cited by applicant

Gries et al.; Importance of Chronopharmacokinetics in Design and Evaluation of Transdermal Drug Delivery Systems; *J Pharmocol Exp Ther*; 285(2); pp. 457-463; May 1998. cited by applicant

Guy; Current status and future prospects of transdermal drug delivery; *Pharm Res*; 13(12); pp. 1765-1769; Dec. 1996. cited by applicant

Halberg et al.; Chronomics: circadian and circaseptan timing of radiotherapy, drugs, calories, perhaps nutraceuticals and beyond; *Journal of Experimental Therapeutics and Oncology*; 3(5); pp. 223-260; Sep. 2003. cited by applicant

He et al.; Autoradiographic analysis of dopamine receptor-stimulated [35S]GTPγS binding in rat striatum; *Brain Research*; 885(1); pp. 133-136; Dec. 1, 2000. cited by applicant

He et al; Autoradiographic analysis of N-methyl-D-aspartate receptor binding in monkey brain: Effects of l-methyl-4-phenyl-1,2,3,6-tetrahydropyridine and levodopa treatment; *Neuroscience*; 99(4); pp. 697-704; Aug. 23, 2000. cited by applicant

Heffner et al.; Feature-level analysis of a novel smartphone application for smoking cessation; *Am. J. Drug Alcohol Abuse*; 41(1); pp. 68-73; Jan. 2015 (Author Manuscript). cited by applicant

Hrushesky; Temporally optimizable delivery systems: sine qua non for the next therapeutic revolution; *J Control Rel*; 19(1-3); pp. 363-368; Mar. 1992. cited by applicant

Hsu et al.; Effect of the D3 dopamine receptor partial agonist BP897 [N-[4-(4-(2-

methoxyphenyl)piperazinyl)butyl]-2-naphthamide] on L-3,4-dihydroxyphenylalanine-induced dyskinesias and parkinsonism in squirrel monkeys; *The Journal of Pharmacology and Experimental Therapeutics*. 311(2); pp. 770-777; Nov. 1, 2004. cited by applicant

Huang et al.; Inhibitory effects of curcumin on in vitro lipoxygenase and cyclooxygenase activities in mouse epidermis; *Cancer Res*; 51(3); pp. 813-819; Feb. 1991. cited by applicant

Hukkanen et al.; Metabolism and disposition kinetics of nicotine; *Pharmacological Reviews*; 57(1); pp. 79-115; Mar. 1, 2005. cited by applicant

Hurley; Growing list of positive effects of nicotine seen in neurodegenerative disorders; *Neurology Today*; 12(2); pp. 37-38; Jan. 19, 2012. cited by applicant

Ingram et al.; Preliminary observations of oral nicotine therapy for inflammatory bowel disease: an open-label phase I-II study of tolerance; *Inflamm Bowel Diseases*; 11(12); pp. 1092-1096; Dec. 1, 2005. cited by applicant

Janson et al.; Chronic nicotine treatment counteracts dopamine D2 receptor upregulation induced by a partial meso-diencephalic hemitransection in the rat; *Brain Res.*; 655(1-2); pp. 25-32; Aug. 29, 1994. cited by applicant

Jarvik et al.; Inhibition of cigarette smoking by orally administered nicotine; *Clinical Pharmacology and Therapeutics*; 11(4); pp. 574-576; Jul. 1, 1970. cited by applicant

Jeyarasasingam et al.; Nitric oxide is involved in acetylcholinesterase inhibitor-induced myopathy in rats; *The Journal of Pharmacology and Experimental Therapeutics*; 295(1); pp. 314-320; Oct. 1, 2000. cited by applicant

Jeyarasasingam et al.; Stimulation of non- $\alpha 7$ nicotinic receptors partially protects dopaminergic neurons from l-methyl-4-phenylpyridinium-induced toxicity in culture; *Neuroscience*; 109(2); pp. 275-285; Jan. 28, 2002. cited by applicant

Jeyarasasingam et al.; Tacrine, a reversible acetylcholinesterase inhibitor, induces myopathy; *Neuroreport*; 11(6); pp. 1173-1176; Apr. 27, 2000. cited by applicant

Kalish et al.; Prevention of contact hypersensitivity to topically applied drugs by ethacrynic acid: potential application to transdermal drug delivery; *J. Controll Rel*; 48(1); pp. 79-87; Sep. 1997. cited by applicant

Kalish et al.; Sensitization of mice to topically applied drugs: albuterol, chlorpheniramine, clonidine and nadolol; *Contact Dermatitis*; 35(2); pp. 76-82; Aug. 1996. cited by applicant

Kelton et al.; The effects of nicotine on Parkinson's disease; *Brain Cognition*; 43(1-3); pp. 274-282; Jun. 2000. cited by applicant

Kennelly; Microcontrollers drive home drug delivery; 3 pgs; posted Jul. 2014; (retrieved Jul. 26, 2016 from the internet: <http://electronicsmaker.com/microcontrollers-drive-home-drug-delivery-2>. cited by applicant

Kiwi Drug; Buy nicorette microtabs; 3 pages; retrieved from the internet (www.kiwidrug.com/search/nicorette_microtabs); on Jul. 26, 2018. cited by applicant

Kotwal; Enhancement of intophoretic transport of diphenhydramine hydrochloride thermosensitive gel by optimization of pH, polymer concentration, electrode design, and pulse rate; *AAPS PharmSciTech*; 8(4); pp. 320-325; Oct. 2007. cited by applicant

Kulak et al.; 5-Iodo-A-85380 binds to $\alpha 5$ -sensitive nicotinic acetylcholine receptors (nAChRs) as well as $\alpha 3 \beta 2^*$ subtypes; *Journal of Neurochemistry*; 81(2); pp. 403-406; Apr. 1, 2002. cited by applicant

Kulak et al.; Declines in different π nicotinic receptor populations in monkey striatum after nigrostriatal damage; *The Journal of Pharmacology and Experimental Therapeutics*; 303(2); pp. 633-639; Nov. 1, 2002. cited by applicant

Kulak et al.; Loss of nicotinic receptors in monkey striatum after l-methyl-4-phenyl-1,2,3,6-tetrahydropyridine treatment is due to a decline in $\alpha 5$ sites; *Molecular Pharmacology*; 61(1); pp. 230-238; Jan. 1, 2002. cited by applicant

Kumar et al.; Levodopa-dyskinesia incidence by age of Parkinson's disease onset; *Movement disorders*; 20(3); pp. 342-344; Mar. 2005. cited by applicant

Kydonieus et al. (Editors); *Biochemical Modulation of Skin Reactions*; CRC Press; Boca Ratan, FL; pp. 9-10; Dec. 1999. cited by applicant

Labrecque, G. et al.; Chronopharmacokinetics; *Pharmaceutical News*; 4(2); pp. 17-21; (year of pub. sufficiently earlier than effective US filing date and any foreign priority date) 1997. cited by applicant

Lai et al.; Long-term nicotine treatment decreases striatal $\alpha 6^*$ nicotinic acetylcholine receptor sites and function in mice; *Molecular Pharmacology*; 67(5); pp. 1639-1647; May 1, 2005. cited by applicant

Lai et al.; Selective recovery of striatal 125I- α -conotoxinMII nicotinic receptors after nigrostriatal damage in monkeys; *Neuroscience*; 127(2); pp. 399-408; Dec. 31, 2004. cited by applicant

Lamberg; Chronotherapeutics: Implications for drug therapy; *American Pharmacy*; NS31(11); pp. 20-23; Nov. 1991. cited by applicant

Langston et al.; Investigating levodopa-induced dyskinesias in the parkinsonian primate; *Annals of Neurology*; 47(4 Suppl 1); pp. S79-S88; Apr. 2000. cited by applicant

Laser et al.; A review of micropumps; *J. of Micromech. And Microeng.*; 14; pp. R35-R64; Apr. 2004. cited by applicant

Lee et al.; A comprehensive review of opioid-induced hyperalgesia; *Pain Physician*; 14; pp. 145-161; Mar. 2011. cited by applicant

Lemay et al.; Lack of efficacy of a nicotine transdermal treatment on motor and cognitive deficits in Parkinson's disease; *Prog Neuropsychopharmacol Biol Psychiatry*; 28(1); pp. 31-39; Jan. 2004. cited by applicant

Lemmer; Clinical Chronopharmacology: The Importance of Time in Drug Treatment, in Ciba Foundation Symposium 183—Circadian Clocks and their Adjustment (eds. Chadwick and Ackrill); John Wiley & Sons, Inc.; pp. 235-253; Apr. 1995. cited by applicant

Lemmer; Implications of chronopharmacokinetics for drug delivery: antiasthmatics, H₂-blockers and cardiovascular active drugs; *Adv Drug Del Rev*; 6(1); pp. 83-100; Jan./Feb. 1991. cited by applicant

Lemmer; The clinical relevance of chronopharmacology in therapeutics; *Pharmacological Research*; 33(2); pp. 107-115; Feb. 1996. cited by applicant

LeWitt et al.; New developments in levodopa therapy; *Neurology*; 62(No. 1, Suppl. 1); pp. S9-S16; Jan. 2004. cited by applicant

Lieber Man; Compressed tablets by direct compression; *Pharmaceutical Dosage forms*; vol. 1, 2nd ed.; Marcel Dekker Inc.; pp. 195-246; (year of pub. sufficiently earlier than effective US filing date and any foreign priority date) 1989. cited by applicant

Lieberman; Compression—coated and layer tablets; *Pharmaceutical Dosage forms*; vol. 1, 2nd ed.; Marcel Dekker Inc.; pp. 266-271; (year of pub. sufficiently earlier than effective US filing date and any foreign priority date) 1989. cited by applicant

Lundblad et al.; Cellular and behavioural effects of the adenosine A_{2a} receptor antagonist KW-6002 in a rat model of l-DOPA-induced Dyskinesia; *Journal of Neurochemistry*; 84(6); pp. 1398-1410; Mar. 2003. cited by applicant

Madandla et al.; Voluntary running provides neuroprotection in rats after 6-hydroxydopamine injection into the medial forebrain bundle; *Metabolic Brain Disease*; 19(1-2); pp. 43-50; Jun. 2004. cited by applicant

Maillefer et al.; A high-performance silicon micropump for an implantable drug delivery system; 12th IEEE Int'l Conf. on Micro Electro Mechanical Systems; MEMS '99; Orlando, FL; pp. 541-546; Jan. 1999. cited by applicant

Matta et al.; Guidelines on nicotine dose selection for in vivo research; *Psychopharmacology (Berl.)*; 190(3); pp. 269-319; Feb. 1, 2007. cited by applicant

Mccallum et al.; Decrease in $\alpha 3^*/\alpha 6^*$ nicotinic receptors in monkey brain after nigrostriatal damage; *Molecular Pharmacology*; 68(3); pp. 737-746; Sep. 2005. cited by applicant

Mccallum et al.; Compensation in pre-synaptic dopaminergic function following nigrostriatal damage in primates; *Journal of Neurochemistry*; 96(4); pp. 960-972; Feb. 1, 2006. cited by applicant

Mccallum et al.; Differential regulation of mesolimbic $\alpha 3/\alpha 6$ $\beta 2$ and $\alpha 4 \beta 2$ nicotinic acetylcholine receptor sites and function after long-term oral nicotine to monkeys; *The Journal of Pharmacology and Experimental Therapeutics*; 318(1); pp. 381-388; Jul. 2006. cited by applicant

Mccallum et al.; Increases in $\alpha 4^*$ but not $\alpha 3^*/\alpha 6^*$ nicotinic receptor sites and function in the primate striatum following chronic oral nicotine treatment; *Journal of Neurochemistry*; 96(4); pp. 1028-1041; Feb. 2006. cited by applicant

McNeil Sweden AB. Package Leaflet: Information for the user. Nicorette Microtab Lemon 2mg sublingual tablets. (This leaflet was last approved in Apr. 16, 2008). retrieved from (

www.lakemedelsverket.se/SPC_PIL/Pdf/enhumpil/Nicorette%20Microtab%20Lemon%202mg%20sublingual%20tablet%20ENG.pdf.) Accessed Aug. 19, 2010. cited by applicant

Medtronic; MiniMed Paradigm® Veo(TM) System (product info.); retrieved May 12, 2015 from the internet: (<http://www.medtronic.co.uk/your-health/diabetes/device/insulin-pumps/paradigm-veo-pump/>); 3 pgs. cited by applicant

Meissner et al.; Priorities in parkinson's disease research; Nature reviews Drug Discovery; 10(5); pp. 377-393; May 1, 2011. cited by applicant

Menzaghi et al.; Interactions between a novel cholinergic ion channel against, SIB-1765F and L-DOPA in the reserpine model of parkinson's disease in rats; Journal of Pharmacology and Experimental Therapeutics; 280(1); pp. 393-401; Jan. 1, 1997. cited by applicant

Merck manual of therapy and diagnosis; 17th edition. Merck Research Laboratories; pp. 1466-1471; (year of pub. sufficiently earlier than effective US filing date and any foreign priority date) 1999. cited by applicant

Meredith et al.; Behavioral models of Parkinson's disease in rodents: a new look at an old problem; Movement Disorders; 21(10); pp. 1595-1606; Oct. 1, 2006. cited by applicant

Meshul et al.; Nicotine Affects Striatal Glutamatergic Function in 6-OHDA Lesioned Rats; Advanced in behavioural Biology, Basal Ganglia VI.; Springer, Boston, MA.; vol. 54; pp. 589-598; (year of pub. sufficiently earlier than effective US filing date and any foreign priority date) 2002. cited by applicant

Meshul et al.; Nicotine alters striatal glutamate function and decreases the apomorphine-induced contralateral rotations in 6-OHDA-lesioned rats; Experimental Neurology; 175(1); pp. 257-274; May 31, 2002. cited by applicant

Molander et al.; Reduction of tobacco withdrawal symptoms with a sublingual nicotine tablet: A placebo controlled study; Nicotinic & Tob. Res.; 2(2); pp. 187-191; May 2000. cited by applicant

Murphy et al.; Transdermal drug delivery systems and skin sensitivity reactions. Incidence and management; Am. J. Clin Dermatol.; 1(6); pp. 361-368; Nov./Dec. 2000. cited by applicant

Mutalik et al.; Glibenclamide transdermal patches: physicochemical, pharmacodynamic, and pharmacokinetic evaluation; J Pharm Sci; 93(6); pp. 1577-1594; Jun. 2004. cited by applicant

Mutalik et al.; Glipizide matrix transdermal systems for diabetes mellitus: preparation, in vitro and preclinical studies; Life Sci; 79(16); pp. 1568-1567; Sep. 2006. cited by applicant

Nakadate et al.; Effects of chalcone derivatives on lipoxygenase and cyclooxygenase activities of mouse epidermis; Prostaglandins; 30(3); pp. 357-368; Sep. 1985. cited by applicant

National Institute of Neurological Disorders and Stroke. Parkinson's Disease: Hope Through Research. 54 pages; Retrieved from the internet (<https://catalog.ninds.nih.gov/pubstatic//15-139/15-139.pdf>) on Jan. 15, 2018. cited by applicant

Newhouse et al.; Nicotine treatment of mild cognitive impairment: a 6-month double-blind pilot clinical trial; Neurology; 78(2); pp. 91-101; Jan. 10, 2012. cited by applicant

Newmark; Plant phenolics as potential cancer prevention agents; Chapter 3 in Dietary Phytochemicals in Cancer Prevention; Chap. 3; Adv. Exp. Med. Biol. 401; pp. 25-34; © 1996. cited by applicant

Ohdo; Changes in toxicity and effectiveness with timing of drug administration: implications for drug safety; Drug Safety; 26(14); pp. 999-1010; Dec. 2003. cited by applicant

Olanow; The scientific basis for the current treatment of Parkinson's disease; Annu. Rev. Med.; 55; pp. 41-60; Feb. 18, 2004. cited by applicant

Olsson et al.; A valve-less planar pump in silicon; IEEE; The 8th International Conference on Solid-State Sensors and Actuators; vol. 2; pp. 291-294, Jun. 1995. cited by applicant

Olsson et al.; An improved valve-less pump fabricated using deep reactive ion etching; Proc. Of the IEEE, 9th Int'l Workshop on MEMS; San Diego, CA; pp. 479-484; Feb. 11-15, 1996. cited by applicant

O'Neill et al.; The role of neuronal nicotinic acetylcholine receptors in acute and chronic neurodegeneration; Current Drug Targets—CNS and Neurological Disorders; 1(4); pp. 399-412; Aug. 1, 2002. cited by applicant

Parkinson Study Group; Levodopa and the progression of Parkinson's disease; N Engl J Med.; 351; pp. 2498-2508; Dec. 9, 2004. cited by applicant

Petzinger et al.; Reliability and validity of a new global dyskinesia rating scale in the MPTP-lesioned non-human primate; Movement Disorders; 16(2); pp. 202-207; Mar. 1, 2001. cited by applicant

Priano et al.; Nocturnal anomalous movement reduction and sleep microstructure analysis in parkinsonian

patients during 1-night transdermal apomorphine treatment; *Neurol Sci.*; 24(3); pp. 207-208; Oct. 2003. cited by applicant

Prosise et al.; Effect of abstinence from smoking on sleep and daytime sleepiness; *Chest*; 105(4); pp. 1136-1141; Apr. 1994. cited by applicant

Quik et al.; Chronic oral nicotine normalizes dopaminergic function and synaptic plasticity in l-methyl-4-phenyl-1,2,3,6-tetrahydropyridine-lesioned primates; *The Journal of Neuroscience*; 26(17); pp. 4681-4689; Apr. 26, 2006. cited by applicant

Quik et al.; Chronic oral nicotine treatment protects against striatal degeneration in MPTP-treated primates; *Journal of Neurochemistry*; 98(6); pp. 1866-1875; Sep. 1, 2006. cited by applicant

Quik et al.; Differential alterations in nicotinic receptor $\alpha 6$ and $\beta 3$ subunit messenger RNAs in monkey substantia nigra after nigrostriatal degeneration; *Neuroscience*; 100(1); pp. 63-72; Sep. 7, 2000. cited by applicant

Quik et al.; Differential declines in striatal nicotinic receptor subtype function after nigrostriatal damage in mice; *Molecular Pharmacology*; 63(5); pp. 1169-1179; May 1, 2003. cited by applicant

Quik et al.; Differential nicotinic receptor expression in monkey basal ganglia; Effects of nigrostriatal damage; *Neuroscience*; 112(3); pp. 619-630; Jul. 5, 2002. cited by applicant

Quik et al.; Expression of D3 receptor messenger RNA and binding sites in monkey striatum and substantia nigra after nigrostriatal degeneration; Effect of levodopa treatment.; *Neuroscience*; 98(2); pp. 263-273; Jun. 30, 2000. cited by applicant

Quik et al.; Increases in striatal preproenkephalin gene expression are associated with nigrostriatal damage but not L-DOPA-induced dyskinesias in the squirrel monkey; *Neuroscience*; 113(1); pp. 213-220; Aug. 2, 2002. cited by applicant

Quik et al.; L-DOPA treatment modulates nicotinic receptors in monkey striatum; *Mol Pharmacol*; 64(3); pp. 619-628; Sep. 2003. cited by applicant

Quik et al.; Localization of nicotinic receptor subunit mRNAs in monkey brain by in situ hybridization; *The Journal of Comparative Neurology*; 425(1); pp. 58-69; Sep. 11, 2000. cited by applicant

Quik et al.; Loss of α -conotoxinMII- and A85380-sensitive nicotinic receptors in Parkinson's disease striatum; *Journal of Neurochemistry*; 88(3); pp. 668-679; Feb. 1, 2004. cited by applicant

Quik et al.; Nicotine administration reduces striatal MPP⁺ levels in mice; *Brain Research*; 917(2); pp. 219-224; Nov. 2, 2001. cited by applicant

Quik et al.; Nicotine and nicotinic receptors; relevance to Parkinson's disease; *Neurotoxicology*; 23(4-5); pp. 581-594; Oct. 2002. cited by applicant

Quik et al.; Nicotine and Parkinson's disease: implications for therapy; *Movement Disorders*; 23(12); pp. 1641-1652; (Author Manuscript); Sep. 1, 2008. cited by applicant

Quik et al.; Nicotine as a potential neuroprotective agent for Parkinson's disease; *Movement disorders*; 27(8); pp. 947-957; Jul. 1, 2012. cited by applicant

Quik et al.; Nicotine neuroprotection against nigrostriatal damage: importance of the animal model; *Trends in Pharmacological sciences*; 28(5); pp. 229-235; May 31, 2007. cited by applicant

Quik et al.; Nicotine reduces levodopa-induced dyskinesias in lesioned monkeys; *Annals of neurology*; 62(6); pp. 588-596; (Author Manuscript); Dec. 1, 2007. cited by applicant

Quik et al.; Nicotinic receptors and Parkinson's disease; *European Journal of Pharmacology*; 393(1); pp. 223-230; Mar. 30, 2000. cited by applicant

Quik et al.; Striatal $\alpha 6^*$ nicotinic acetylcholine receptors: Potential targets for Parkinson's disease therapy; *The Journal of Pharmacology and Experimental Therapeutics*; 316(2); pp. 481-489; Feb. 1, 2006. cited by applicant

Quik et al.; Subunit composition of nicotinic receptors in monkey striatum: Effect of treatments with l-methyl-4-phenyl-1,2,3,6-tetrahydropyridine or L-DOPA; *Molecular Pharmacology*; 67(1); pp. 32-41; Jan. 2005. cited by applicant

Quik et al.; Vulnerability of ¹²⁵I- α -conotoxin MII binding sites to nigrostriatal damage in monkey; *The Journal of Neuroscience*; 21(15); pp. 5494-5500; Aug. 1, 2001. cited by applicant

Quik; Smoking, nicotine and Parkinson's disease; *Trends in Neurosciences*; 27(9); pp. 561-568; Sep. 2004. cited by applicant

Reinberg et al.; Circadian rhythms, jet lag, and chronobiotics: An overview; *Chronobiology International*; 11(4); pp. 253-265; Aug. 1994. cited by applicant

Reinberg; Concepts of Circadian Chronopharmacology; *Annals of the New York Academy of Sciences*; 618 (Temporal Control of Drug Delivery); pp. 102-115; Feb. 1991. cited by applicant

Rueter et al.; ABT-089: Pharmacological properties of a neuronal nicotinic acetylcholine receptor agonist for the potential treatment of cognitive disorders; *CNS Drug Reviews*; 10(2); pp. 167-182; Jun. 1, 2004. cited by applicant

Samii et al.; Parkinson's disease; *The Lancet*; 363(9423); pp. 1783-1793; May 29, 2004. cited by applicant

Savitt et al.; Diagnosis and treatment of Parkinson disease: molecules to medicine; *The Journal of Clinical Investigation*; 116(7); pp. 1744-1754; Jul. 3, 2006. cited by applicant

Schapira; Disease modification in Parkinson's disease; *The Lancet Neurology*; 3(6); pp. 362-368; Jun. 30, 2004. cited by applicant

Schneider et al.; Effects of SIB-1508Y, a novel neuronal nicotinic acetylcholine receptor agonist, on motor behavior in parkinsonian monkeys; *Movement Disorders*; 13(4); pp. 637-642; Jul. 1, 1998. cited by applicant

Schneider et al.; Effects of the nicotinic acetylcholine receptor agonist SIB-1508Y on object retrieval performance in MPTP-treated monkeys: Comparison with levodopa treatment; *Annals of Neurology*; 43(3); pp. 311-317; Mar. 1, 1998. cited by applicant

Schober et al.; Classic toxin-induced animal models of Parkinson's disease: 6-OHDA and MPTP; *Cell and Tissue Research*; 318(1); pp. 215-224; Oct. 1, 2004. cited by applicant

Shin et al.; Enhanced bioavailability of triprolidine from the transdermal TPX matrix system in rabbits; *Int. J. Pharm.*; 234(1-2); pp. 67-73; Mar. 2002. cited by applicant

Silver et al.; Transdermal nicotine and haloperidol in Tourette's disorder: a double-blind placebo-controlled study; *Journal of Clinical Psychiatry*; 62(9); pp. 707-714; Sep. 1, 2001. cited by applicant

Singer et al.; Nightmares in patients with Alzheimer's disease caused by donepezil: Therapeutic effect depends on the time of intake; *Nervenarzt*; 76(9); pp. 1127-1129; Sep. 2005 (Article in German w/ Eng. Summary). cited by applicant

Star Micronics Co., Ltd; Prototype Diaphragm Micro Pump SDMP305 (specifications); retrieved May 12, 2015 from the internet archive as of Jul. 2006 (<http://www.star-m.jp/eng/products/develop/de07.htm>); 3 pgs. cited by applicant

Stocchi et al.; Motor fluctuations in levodopa treatment: clinical pharmacology; *European Neurology*; 36(Suppl 1); pp. 38-42; Jan. 1996. cited by applicant

Strong et al.; Genotype and smoking history affect risk of levodopa-induced dyskinesias in parkinson's disease; *Movement Disorders*; 21(5); pp. 654-659; May 1, 2006. cited by applicant

Thiele et al. (Ed.); Oxidants and Antioxidants in Cutaneous Biology: Current Problems in Dermatology (Book 29); S. Karger; 196 pgs., Feb. 2001. cited by applicant

Togasaki et al.; Dyskinesias in normal squirrel monkeys induced by nomifensine and levodopa; *Neuropharmacology*; 48(3); pp. 398-405; Mar. 31, 2005. cited by applicant

Togasaki et al., Levodopa induces dyskinesias in normal squirrel monkeys; *Annals of Neurology*; 50(2); pp. 254-257; Aug. 1, 2001. cited by applicant

Togasaki et al.; The Webcam system: A simple, automated, computer-based video system for quantitative measurement of movement of nonhuman primates; *Journal of Neuroscience Methods*; 145(1); pp. 159-166; Jun. 30, 2005. cited by applicant

Tolosa et al.; Antagonism by piperidine of levodopa effects in Parkinson disease; *Neurology*; 27(9); pp. 875-877; Sep. 1, 1977. cited by applicant

U.S. Department of Health and Human Services, Food and Drug Administration, Center for Drug Evaluation and Research (CDER); Guidance for industry: Abuse-deterrent opioids—Evaluation and labeling; 24 pages; retrieved from the internet (<http://www.fda.gov/downloads/drugs/guidancecomplianceregulatoryinformation/guidances/ucm344743.pdf>); Jan. 2013. cited by applicant

United States of America VA/DoD; Tapering and discontinuing opioids; 2 pages; retrieved from the internet (<http://www.healthquality.va.gov/guidelines/Pain/cot/OpioidTaperingFactSheet23May2013v1.pdf>); on Sep. 1, 2016. cited by applicant

Vierregge et al.; Transdermal nicotine in PD: A randomized, double-blind, placebo-controlled study; Neurology; 57(6); pp. 1032-1035; Sep. 25, 2001. cited by applicant

Villafane et al.; Long-term nicotine administration can improve Parkinson's disease: report of a case after three years of treatment; Revista Neurologica Argentina; 27(2); pp. 95-97; (year of pub. sufficiently earlier than effective US filing date and any foreign priority date) 2002. cited by applicant

Warburton et al.; Facilitation of learning and state dependency with nicotine; Psychopharmacology; 89(1); pp. 55-59; May 1, 1986. cited by applicant

Wermuth et al.; Glossary of terms used in medicinal chemistry Pure & Appl. Chem., vol. 70(5); 1129-1143; 1998 AC recommendations 1998); Pure and Applied Chemistry; 70(5); pp. 1129-1143; Jan. 1998. cited by applicant

Wesnes et al.; Effects of scopolamine and nicotine on human rapid information processing performance; Psychopharmacology; 82(3); pp. 147-150; Sep. 1, 1984. cited by applicant

Westman et al.; Oral nicotine solution for smoking cessation: a pilot tolerability study; Nicotine and Tobacco Research; 3(4); pp. 391-396; Nov. 1, 2001. cited by applicant

Wille et al.; cis-urocanic Acid Induces Mast Cell Degranulation and Release of Preformed TNF-alpha: A Possible Mechanism Linking UVB and cis-urocanic Acid to Immunosuppression of Contact Hypersensitivity; Skin Pharm Appl Skin Physiol; 12(1-2); pp. 18-27; Jan. 1999. cited by applicant

Wille et al.; Inhibition of irritation and contact hypersensitivity by ethacrynic acid; Skin Pharm Appl Skin Physiol; 11(4-5); pp. 279-288; Jul. 1998. cited by applicant

Wille et al.; Inhibition of Irritation and Contact Hypersensitivity by Phenoxyacetic Acid Methyl Ester in Mice; Skin Pharm Appl Skin Physiol; 13(2); pp. 65-74; Mar. 2000. cited by applicant

Wille et al.; Several different ion channel modulators abrogate contact hypersensitivity in mice; Skin Pharm Appl Skin Physiol; 12(1-2); pp. 12-17; Jan. 1999. cited by applicant

Wille, J.; Novel topical delivery system for plant derived hydrophobic anti-irritant active (presentation abstract No. 273); 226th ACS National Meeting; New York, NY; Sep. 7-11, 2003. cited by applicant

Wille; In Closing: an editorial on Plant-Derived Anti-irritants. Cosmetics & Toiletries, 118 (8), Aug. 2003. cited by applicant

Wille; Novel plant-derived anti-irritants; (presented Dec. 5-6, 2002 at the 2002 Ann. Scientific Mtg. & Tech. Showcase); J. Cosmet. Sci.; 54; pp. 106-107; Jan./Feb. 2003. cited by applicant

Wille; Thixogel: Novel topical delivery system for hydrophobic plant actives; in Rosen (Ed.) Delivery System Handbook for Personal Care and Cosmetic Products; 1st Ed.; ISBN: 978-0-8155-1504-3; pp. 762-794; Sep. 2005. cited by applicant

Youan; Chronopharmaceutics: gimmick or clinically relevant approach to drug delivery?; J Cont Rel; 98(3); pp. 337-353; Aug. 2004. cited by applicant

Yun et al.; A distributed memory MIMD multi-computer with reconfigurable custom computing capabilities; IEEE; Proc. Int'l. Conf. on Parallel and Distributed Systems; pp. 8-13; Dec. 10-13, 1997. cited by applicant

Zubieta et al.; Placebo effects mediated by endogenous opioid activity on mu-opioid receptors; 25(34); pp. 7754-7762; Aug. 24, 2005. cited by applicant

Netzel et al.; U.S. Appl. No. 17/815,879 entitled "Drug Delivery methods and systems," filed Jul. 28, 2022. cited by applicant

Dipierro et al.; U.S. Appl. No. 17/936,750 entitled "Optimized bio-synchronous bioactive agent delivery system," filed Sep. 29, 2022. cited by applicant

Johnston et al.; U.S. Appl. No. 17/703,910 entitled "Transdermal drug delivery devices and methods," filed Mar. 24, 2022. cited by applicant

Tong et al.; U.S. Appl. No. 18/178,442 entitled "Drug Delivery methods and systems," filed Mar. 3, 2023. cited by applicant

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Background/Summary

CROSS REFERENCE TO RELATED APPLICATIONS (1) This application is a U.S. National Phase Application Under 35 U.S.C. § 371 of International Application No. PCT/US2019/061761, filed on Nov. 15, 2019, titled “THERMALLY REGULATED TRANSDERMAL DRUG DELIVERY SYSTEM,” now International Publication No. WO 2020/102695, which claims priority to U.S. Provisional Application No. 62/768,768, filed on Nov. 16, 2018, titled “THERMALLY REGULATED TRANSDERMAL DRUG DELIVERY SYSTEM,” each of which is herein incorporated by reference.

INCORPORATION BY REFERENCE

(1) All publications and patent applications mentioned in this specification are herein incorporated by reference in their entirety to the same extent as if each individual publication or patent application was specifically and individually indicated to be incorporated by reference.

FIELD

(2) This disclosure is related generally to delivery of drugs and other active agents to a subject. More specifically, this disclosure relates to delivery of drugs and other active agents to skin or mucosa of a subject.

BACKGROUND

(3) Cigarette smoking causes more than 480,000 deaths each year in the United States alone. Smoking increases the risk of coronary heart disease, stroke, lung cancer and other diseases up to 25 times. Despite the availability of pharmaceutical agents and various patches to help people stop smoking, many people are unable to give up smoking. This is mostly due to the strong cravings for smoking that smokers feel.

(4) Further, approximately one million people in the United States alone have Parkinson's disease. Parkinson's disease is a degenerative disease of the nervous system with no known cure. The symptoms of Parkinson's disease include tremor, joint rigidity, balance problems, and slow movement. The symptoms get progressively worse over time.

(5) Various systems have been developed to deliver pharmaceutical agents to a subject, such as those who wish to stop smoking or who suffer from Parkinson's disease. However, these existing systems suffer from various limitations and problems. For example, injections of a pharmaceutical drug is painful. Orally ingested drugs may not be available when needed and/or may dissolve in the acid of the stomach.

(6) Transdermal drug delivery, i.e., using the skin of the body to deliver pharmaceutical agents, can be advantageous, as transdermal delivery prevents pharmaceutical agents from being digested in the stomach and eliminates pain from direct injection. However, the barrier function of the skin can make it difficult to get pharmaceutical drugs transported across the skin and into the body. Thus, for example, in passive transdermal drug delivery, a drug delivery patch is placed on the skin, and the rate at which the patient receives the drug is limited by how quickly (or slowly) the drug can diffuse across the skin. Additionally, such passive transdermal delivery systems often require frequent replacement of the transdermal patch. Further, passive transdermal systems often do not provide customizable drug delivery, making them less suitable for clinical applications that require a fast or controlled onset, such as a quick delivery of nicotine to decrease cigarette cravings or a timely delivery of Parkinson's drugs to counteract Parkinson's symptoms.

(7) Described herein are systems and methods to address limitations of existing transdermal drug delivery systems.

SUMMARY OF THE DISCLOSURE

(8) Described herein are drug and other active agent delivery systems and devices for delivering active agents to skin or mucosa of a subject, and methods of delivering such active agents, and in particular, for controlling the timing and amount of drug and other active agent delivery.

(9) In general, in one embodiment, a transdermal delivery system includes an active agent integrated with or fluidically connected to a transdermal membrane and a temperature control element. The transdermal membrane is configured to allow the active agent to flow therethrough to skin of a subject. The temperature control element is configured to heat and/or cool a portion of the transdermal delivery system so as to provide pulsatile delivery of the active agent through the transdermal membrane.

(10) This and other embodiments can include one or more of the following features. The temperature control element can include an electromagnetic energy source. The temperature control element can include a resistive element. The temperature control element can include inductive coil or an electromagnet. The temperature control element can include a coolant or heat sink. The portion of the transdermal membrane can include a polymer configured to be heated or cooled to thereby change active agent flow. The portion of the transdermal membrane can include a glass transition polymer configured to be heated or cooled to thereby change active agent flow. The portion of the transdermal membrane can include a magnetic nanoparticle configured to be heated or cooled to thereby change active agent flow. The active agent can include nicotine or a nicotine agonist. The active agent can include a Parkinson's disease treatment. The active agent can include at least one of benztropine, carbidopa, dopamine, a dopamine analog, a dopamine antagonist, a dopamine agonist, entacapone, levodopa (L-dopa), pramipexole, rasagiline, ropinirole, rotigotine, safinamide, selegiline, both carbidopa and levodopa, trihexyphenidyl and tolcapone. The transdermal delivery system can further include an adhesive in the transdermal membrane configured to adhere the transdermal membrane to the subject. The transdermal delivery system can further include an adhesive in the transdermal membrane, and the adhesive can contain the active agent. The transdermal delivery system can further include a temperature sensor configured to measure the temperature of at least one of the temperature control element, the portion of the transdermal delivery system, or the transdermal membrane. The transdermal delivery system can further include a reservoir of active agent. The transdermal delivery system can further include a power source configured to provide power to the temperature control element. The transdermal delivery system can further include a circuit board. The transdermal delivery system can further include a microcontroller configured to control delivery of a stimulus from the temperature control element. The transdermal delivery system can further include a communication element configured to receive or transmit data and the communication element can include Bluetooth or WiFi.

(11) In general, in one embodiment, a method of transdermally delivering an active agent, includes (1) actively heating or cooling a portion of a transdermal delivery system for a first amount of time; and (2) after the first amount of time, actively heating or cooling the portion of the transdermal delivery system for a second amount of time. The actively heating or cooling steps provide pulsatile delivery of active agent from a transdermal membrane of the transdermal delivery system to skin of a patient.

(12) This and other embodiments can include one or more of the following features. The heating or cooling steps can include delivering heat to the portion of the transdermal delivery system. The actively heating or cooling steps can include removing heat from the portion of the transdermal delivery system. At least one of the actively heating or cooling steps can include delivering electromagnetic radiation to the portion of the transdermal delivery system. The method can further include repeating the actively heating or cooling steps at least once. The method can further include repeating the actively heating or cooling steps at least twice. The portion of the transdermal delivery system can be increased in temperature by at least 4° C. during the actively heating and cooling steps. The method can further include actively heating skin of the subject adjacent the transdermal membrane by at least 3° C. during the actively heating or cooling steps. At least one of the actively heating or cooling steps can be performed while the subject is sleeping.

(13) In general, in one embodiment, a transdermal delivery system includes an active agent, a transdermal membrane, and a temperature control element. The active agent is integrated with or fluidically connected to a transdermal membrane. The transdermal membrane is configured to allow the active agent to flow therethrough to skin of a subject. The temperature control element is configured to provide a stimulus to a portion of the transdermal delivery system so as to provide pulsatile delivery of the active agent through the transdermal membrane to the skin of the subject.

(14) This and other embodiments can include one or more of the following features. The system can further include a reactive material configured to prevent the active agent from flowing across the transdermal membrane until the stimulus is applied. The reactive material can include an epoxy, a polyethylene, a polymethacrylate, a polypropylene, a polypropylene glycol, a polyvinylacetate, a polystyrene, a polytetrafluoroethylene, a poly(bisphenol A carbonate), a poly(ethylene terephthalate), a polylactic acid (PLA), a polyglycolic acid (PGA), or a polyurethane. The reactive material can include a polymer, a hydrogel, a solvent, gold covered nanoparticles, or magnetic nanoparticles.

Description

BRIEF DESCRIPTION OF THE DRAWINGS

(1) The novel features of the invention are set forth with particularity in the claims that follow. A better understanding of the features and advantages of the present invention will be obtained by reference to the following detailed description that sets forth illustrative embodiments, in which the principles of the invention are utilized, and the accompanying drawings of which:

(2) FIG. 1 shows a transdermal delivery system with temperature control for controlling active agent delivery to a subject.

(3) FIG. 2 shows another transdermal active agent delivery system with temperature control for controlling active agent delivery to a subject.

(4) FIG. 3 shows a transdermal delivery system with temperature control and a complementary transdermal membrane for controlling active agent delivery to a subject.

(5) FIG. 4 shows results of nicotine delivery after heating a matrix-type transdermal membrane to deliver nicotine through the transdermal membrane.

DETAILED DESCRIPTION

(6) Transdermal delivery systems, devices, and methods are described herein for delivering an active agent, such as a drug, to a subject. For example, described herein are transdermal delivery systems configured to control delivery of one or more active agents to a skin or mucosal surface of a subject and methods of using such systems. The active agent may be any agent useful for furnishing pharmacological activity or other effect in the diagnosis, cure, mitigation, treatment, or prevention of a disorder, a disease, a symptom, a syndrome and/or to affect any structure or any function of a human or other body. The active agent may act on the skin or mucosal surface and/or move into the blood stream, where it may travel systemically.

(7) In some embodiments, the transdermal delivery systems described herein can be used to provide an active agent at a steady state level in the bloodstream. For example, the transdermal delivery systems can be used to provide an extended release medicine to provide a steady state of relief to manage chronic pain. In other embodiments, the transdermal drug delivery systems described herein can be used to provide an active agent such that the level of active agent varies in the bloodstream change over time. For example, the transdermal delivery systems can be used to deliver nicotine at particular times for smoking cessation. A habitual smoker who is trying to give up smoking can have very powerful cravings for cigarettes at particular times, such as upon waking up after a night's sleep and after eating a meal. Since a dose of nicotine can help manage intense cravings, it may be advantageous to give such a person a dose of nicotine only at certain times, e.g., when the cravings are strongest.

(8) In general, the transdermal delivery systems described herein include an active agent integrated with or fluidically connected to a transdermal membrane, a transdermal membrane configured to allow active agent to flow therethrough, and a temperature control element configured to stimulate a portion of the transdermal delivery system (e.g., a transdermal membrane or an active agent therein) so as to control the timing and/or amount of delivery of the active agent through the transdermal membrane. In some embodiments, the temperature control element is a heating and cooling element configured to heat and/or cool a portion of the transdermal delivery membrane so as to provide pulsatile delivery of the active agent through the transdermal membrane. The transdermal systems described herein can be configured to control an amount and a timing of active agent delivery through the transdermal delivery system.

(9) The transdermal systems described herein can be configured to release an active agent through the transdermal membrane to a subject in a pulsatile flow. As a result, the flux (the amount of active agent that crosses the skin per unit area per unit time) can be controlled and tuned up and down. For example, the flux can be controlled so as to deliver active agent for 1 delivery cycle (with 1 peak and 1 valley), 2 delivery cycles, 3 delivery cycles, or more than 3 delivery cycles. Each delivery cycle may be from a few minutes to many hours or even a day or more. Further, different delivery cycles may advantageously last for different lengths of time. When fluid is delivered during a delivery cycle, it may be referred to a pulse of active agent delivery. The transdermal delivery systems described herein can control the period of time over which a desired amount of an active agent flows across a transdermal membrane for any given cycle.

(10) In any of the delivery systems described herein, the concentration of active agent in the transdermal membrane available for delivery to a subject may be alternately increased and decreased, allowing for alternately increased and decreased flow (including down to no flow) of active agent across the transdermal membrane to the subject. In some embodiments, the transdermal systems described herein may be able to alternately heat and cool a portion of the transdermal delivery system to increase and decrease the concentration of active agent in the transdermal membrane.

(11) The drug delivery systems and methods described herein can heat, cool, or otherwise stimulate the skin to control the flux of active agent therethrough. That is, once flow of active agent has reached skin of a subject, flow of the active agent across the skin of the subject is modulated by characteristics of the skin, such as skin composition and skin temperature. In some embodiments, a portion of the subject's skin, such as skin adjacent to the transdermal membrane, may be stimulated, such as by heating and cooling, to modulate active agent delivery across the skin.

(12) FIG. 1 shows a transdermal delivery system **20** with temperature control element **3**, power source **4**, and transdermal membrane **10**. Active agent **2** is integrated with transdermal membrane **10**. Adhesive matrix **14** adheres transdermal delivery system **20** to skin **1** of the subject. Transdermal membrane **10** is configured to allow the active agent **2** in the transdermal membrane **10** to flow therethrough to skin **1** of the subject. Power source **4** is configured to provide power to the temperature control element **3** and may be a rechargeable or non-rechargeable battery such as a lithium metal, lithium ion, nickel metal hydride, or nickel cadmium battery. If rechargeable, power source **4** may be rechargeable using a wired or wireless mechanism, such as using wireless inductive recharging from a smart phone.

(13) Temperature control element **3** can be positioned at least partially within transdermal membrane **10**. The temperature control element **3** may include one or more resistive heating elements, infrared light sources, or chemicals for exothermic chemical reactions. In some embodiments, temperature control element **3** is configured to generate heat (e.g., kinetic energy) or other energy. Further, temperature control element **3** can deliver heat, coolant, or other energy to the transdermal membrane **10** and thereby control (increase, decrease or maintain) a temperature of the transdermal membrane **10** and/or active agent **2**. The temperature control element **3** may increase temperature of a portion of the transdermal membrane **10** (and/or the active agent **2**) by at least 1° C., at least 2° C., at least 3° C., at least 4° C., at least 5° C., at least 6° C., at least 7° C., at least 8° C., at least 9° C. or at least 10° C. or by not more than 1° C., not more than 2° C., not more than 3° C., not more than 4° C., not more than 5° C., not more than 6° C., not more than 7° C., not more than 8° C., not more than 9° C., or not more than 10° C., or any value between these numbers (such as at by at least 2° C. and not more than 5° C.) by the applied heat or other energy. Further, the applied heat or other energy from the temperature control element **3** can act on the transdermal membrane **10** (and/or a component in the transdermal membrane **10**, such as the active agent **2**) to allow or increase active agent delivery through transdermal membrane **10** to skin **1**. For example, the applied heat or other energy can increase active agent diffusion and movement within transdermal membrane **10** and/or can increase the pore size of the transdermal membrane **10** to enhance flow therethrough.

(14) In some embodiments, energy delivery to a portion of the transdermal delivery system **20**, such as to the transdermal membrane **10**, may alternate between two (or more) conditions, such as an “on” condition and an “off” condition. For example, a first delivery condition may have higher level of heat delivery (e.g., a delivery at a higher temperature and/or delivery for a longer time), and a second

delivery condition may have a lower level (including no) heat delivery (e.g., lower temperature and/or for a longer time). A higher level of heat delivery can be sufficiently high so as to enable flow of a sufficient amount of active agent across the transdermal membrane **10** to skin **1** to have a therapeutic, prophylactic, or other effect. In contrast, a lower heat delivery state can be sufficiently low (or be off) such that there is insufficient (or no) flow of active agent across transdermal membrane **10** and therefore no therapeutic or prophylactic effect. In some embodiments, the transdermal delivery system **20** may alternate repeatedly between higher and lower heat delivery conditions to create pulsatile active agent delivery (e.g., a plurality of pulses of active agent delivery). For example, the transdermal delivery system **20** may produce 1 pulse, 2 pulses, 3 pulses, 4 pulses, 5 pulses, 6 pulses, 7 pulses, 8 pulses, 9 pulses, or 10 pulses or more than 10 pulses.

(15) In some embodiments, a lower (or no) heat delivery state may result from temperature control element **3** being off or inactive, thereby reducing or eliminating heat delivery to a portion of the transdermal delivery system **20**. In some embodiments, temperature control element **3** may be configured (e.g., may have a refrigerant or act as a heat sink) to actively decrease a temperature of a portion of the transdermal delivery system **20**. The lower (or no heat) delivery or active cooling by the temperature control element **3** may reduce the temperature of a portion of transdermal delivery system **20** (relative to the temperature of that portion of the transdermal delivery system **20** or relative to the temperature of that portion of the transdermal delivery system **20** that has previously been heated) by at least 1° C., at least 2° C., at least 3° C., at least 4° C., at least 5° C., at least 6° C., at least 7° C., at least 8° C., at least 9° C., or at least 10° C. or by not more than 1° C., not more than 2° C., not more than 3° C., not more than 4° C., not more than 5° C., not more than 6° C., not more than 7° C., not more than 8° C., not more than 9° C., or not more than 10° C. or any value between these numbers (such as by at least 2° C. and not more than 5° C.). In some embodiments, the temperature of a portion of the transdermal delivery system **20** in place on the user's skin may be about 30° C. (e.g., between 29.5° C. and 30.5° C.), about 31° C., about 32° C., about 33° C., about 34° C., about 35° C., about 36° C., about 37° C., about 38° C., or about 39° C. (e.g., $\pm 0.5^\circ\text{C}$). As the temperature of the user's skin may be cooler than the user's body or blood temperature (e.g., about 32° C., about 33° C., or about 34° C. rather than about 36° C. or about 37° C.), the baseline temperature of part of the transdermal delivery system **20** in place on the user may be about 32° C., about 33° C., or about 34° C., and the transdermal delivery system **20** may be configured to not deliver active agent at such temperatures.

(16) In some embodiments, the transdermal delivery system **20** may be configured to heat or cool the user's skin instead of, or in addition to, heating or cooling a portion of the transdermal delivery system **20**. Such heating or cooling may alter active agent delivery through the skin, such as allowing or reducing active agent delivery. Thus, the temperature control element **3** may be configured to deliver heating or cooling or other energy to skin **1** of the subject to heat or cool the skin **1**. The temperature control element **3** may deliver heating or cooling or other energy to skin **1** indirectly (e.g., through conduction, such as a heated or cooled transdermal membrane) directly or (e.g., via infrared energy delivery to heat the skin). Heating of the skin may allow the flux (amount of active agent that crosses the skin per unit area per unit time) to exceed that which is typically observed in transdermal delivery (e.g., in passive transdermal delivery systems) and/or that which is achieved without heating. This increased flux may be useful, for example, for getting active agents that are not readily deliverable by other transdermal systems delivered across the skin or for controlling pulses of active agent delivery. In some embodiments, both the transdermal membrane **10** and the skin **1** can be heated in response to the applied heat or other energy.

(17) In some embodiments, the upper limit, lower limit, or range of desired temperatures for the portion of transdermal delivery system **20** to be heated or cooled (and/or the temperature of the skin) may be tuned to the particular system for desired efficacy, comfort, and safety. That is, too high of a temperature may be uncomfortable or may burn a subject. Too low of a temperature may not facilitate sufficient delivery of a desired amount of active agent. For example, a system **20** configured for delivering a larger amount of an active agent through the front of the arm (dorsal arm skin) may have a higher maximum temperature than a system **20** for delivering a smaller amount of an active agent through the neck. The temperature of skin **1** may be heated so that it is at least 32° C., at least 33° C., at

least 34° C., at least 35° C., at least 36° C., at least 37° C., at least 38° C., at least 39° C., or at least 40° C. or to a temperature not more than 40° C., 39° C., 38° C., 37° C., 36° C., 35° C., or 34° C. or any temperature in between these numbers. The temperature of skin **1** may be increased by at least 1° C., at least 2° C., at least 3° C., at least 4° C., at least 5° C., at least 6° C., at least 7° C., at least 8° C. or by not more than 1° C., not more than 2° C., not more than 3° C., not more than 4° C., not more than 5° C., not more than 6° C., or not more than 7° C., or any value between these numbers (such as at by least 2° C. and by not more than 5° C.) by the applied heat or other energy.

(18) The transdermal delivery system **20** may be configured to provide pulsatile delivery of active agent. Pulsatile delivery includes an “on” period of active agent delivery and an “off” period without or with a low level of active agent delivery. Temperature control element **3** may alternate between a period of heating and a period of non-heating (or cooling). Alternating a period of heating for delivering active agent with a period of non-heating (or cooling) may lead to active agent delivery with 1 pulse, 2 pulses, 3 pulses, 4 pulses, or 5 or more than 5 pulses of active agent delivery for pulsatile delivery of the active agent **2**. A delivery window during which a therapeutically effective or otherwise desired dosage of an active agent is delivered from the transdermal delivery system **20** may last at least 10 minutes, at least 30 minutes, at least 1 hour, at least 2 hours, at least 3 hours, at least 4 hours, at least 6 hours, at least 8 hours, at least 10 hours, at least 12 hours, or at least 18 hours, or less than 10 minutes, less than 30 minutes, less than 1 hour, less than 1 hour, less than 2 hours, less than 3 hours, less than 4 hours, less than 6 hours, less than 8 hours, less than 10 hours, less than 12 hours, or less than 18 hours. An “off” period during which active agent delivery is “off” (or an ineffective or untherapeutic amount of active agent is delivered) may last at least 10 minutes, at least 30 minutes, at least 1 hour, at least 2 hours, at least 3 hours, at least 4 hours, at least 5 hours, at least 6 hours, at least 10 hours, at least 12 hours, at least 18 hours, at least 1 day or less than 10 minutes, less than 30 minutes, less than 1 hour, less than 2 hours, less than 3 hours, less than 4 hours, or less than 6 hours, less than 10 hours, less than 12 hours, less than 18 hours or less than 1 day or anything in between these amounts. In some embodiments, active agent delivery is “off” most of the time when a subject is planning on sleeping and then is on before the subject plans to wake up, such as one hour before the subject plans to wake up. Active agent delivery may be “on” for several pulses based on a subject's planned activities such as before the subject plans to wake up (e.g., starting one hour before), at a planned lunch time (e.g., starting 30 minutes before) and at a planned dinner time (e.g., starting 30 minutes before). The amount of active agent delivery during “on” periods may be substantially the same for each period or may be different for the different periods. For example, greater amounts of active agent may be delivered during the wake up period than at the lunch period.

(19) FIG. **1** also shows flexible circuit board **5** and microcontroller **6** in transdermal delivery system **20** useful for controlling the active agent delivery system. Microcontroller **6** may include a processor, memory, and input/output peripheral for controlling transdermal “on” and “off” parameters.

Microcontroller **6** be preprogrammed to control temperature control element **3** for turning heat or other energy on and off. Microcontroller **6** may control time of heating/no (or little) heating for temperature control element **3** as well as the number of pulses of heat/no (or little) heat. Microcontroller **6** may also control the temperature of heating/no (or little) heating. The transdermal delivery system **20**, such as temperature control element **3** or transdermal membrane **10**, may include a temperature sensor, such as a thermometer for measuring temperature. The temperature sensor may provide feedback to the system to stop (or reduce) heating or to start heating once a portion of the system reaches a particular temperature. The limit may be useful for limiting an amount of active agent delivery (e.g., to prevent an overdose or more active agent than desired) or to prevent discomfort or prevent a burn to the subject.

(20) FIG. **1** also shows communication module **7** in transdermal delivery system **20**. Communication module **7** may be configured to connect and communicate with an external data source, such as a smart phone, a tablet, a computer, a server, or the internet (a TCP/IP network). Communication may be by wire or may be wireless such as using Bluetooth, Bluetooth Low Energy, WiFi, WiMAX, or Zigbee technology. Communication module **7** may send data from the transdermal delivery system **20** to the external source, such as data about transdermal membrane usage, one or more temperatures of one or more portions of transdermal delivery system **20**, or duration of particular temperatures in one or more

portions of the transdermal delivery system **20**. Communication module **7** may receive data from a user, such as data for controlling: active agent delivery time and duration, temperature changes and duration of heating, and/or number of active agent delivery pulses. In some embodiments, the user may send signals to communication module **7** from a smart phone.

(21) The transdermal membrane **10** may be adhered or configured to adhere to the skin **1** or mucosal surface of a subject, such as through a removable adhesive matrix **14**. In some embodiments, the adhesive matrix **14** may be sufficiently strong such that it is able to hold the transdermal delivery system **20** in place on skin **1** with or without a band or other fastener. The adhesive matrix **14** may include an acrylate, a methacrylate, or an epoxy diacrylate for adhering. The adhesive matrix **14** may be configured to substantially seal the transdermal membrane **10** of the system to the subject's skin or mucosa. The adhesive matrix **14** may be configured to minimize or prevent significant entry of air or contaminants from the environment to the transdermal membrane **10**. In one embodiment, the adhesive matrix **14** may be configured to prevent a solvent (e.g., in the transdermal membrane) from unwanted evaporation during transdermal system use. In some embodiments, the adhesive matrix **14** contains the active agent **2**. For example, active agent **2** may be dispersed or dissolved in part or all of the adhesive matrix **14**. The adhesive matrix **14** may hold part(s) of the transdermal system **20** together, and/or contain the active agent **2** and/or hold the transdermal system **20** to the subject.

(22) FIG. **1** also shows flexible circuit board **5** and microcontroller **6** for controlling the active agent delivery system. Microcontroller **6** may be preprogrammed to control periods of heating/no heating of a portion of the transdermal delivery system **20**. Microcontroller **6** may also or instead be programmed to control periods of heating/no heating from received instructions, such as from a user interface on the transdermal delivery system **20** or from an external communication, such as instructions delivered using communication module **7**.

(23) By controlling periods and levels of heating/no heating (cooling), a pulsatile delivery can be achieved from the transdermal membrane **10**. The control mechanisms described herein allow the delivery of the active agent **2** to be controlled to meet the clinical need for variable active agent delivery, such as increasing therapeutic or other effect when a symptom being targeted is problematic and not at times when the symptom is not as problematic. For example, smokers often experience a craving for a cigarette at particular times, such as intense cravings when first awaking after sleep. A controlled dose of nicotine given to a subject at the right time may control such a craving and keep a person from smoking upon waking. Using the transdermal delivery system **20** with heating and cooling as described herein, for example, allows a smoker to receive a nicotine dose when their cravings are normally particularly troublesome. A dose of active agent **2** may be given when needed or in anticipation of a need. A dose of the active agent **2** may be given to a subject when a subject is awake or when they are asleep. For example, some symptoms or health issues (e.g., cigarette cravings, heart attacks, or migraine headaches) can be especially troublesome in the morning or upon awakening. Using the transdermal delivery systems described herein, a dose of the active agent **2** can be started before the subject plans to wake up, such as at least 15 minutes, at least 30 minutes, at least 45 minutes, at least 1 hour, at least 2 hours, or at least 3 hours before the subject plans to wake up.

(24) Additionally, by tuning the properties of the transdermal system **20** so that delivery of the active agent **2** is lessened or essentially non-existent in the absence of applied heat, no or subclinical levels of active agent dosages can be delivered, effectively turning off the active agent effects in those periods. This may be achieved with an active agent/excipient/matrix adhesive such that diffusion of the active agent is minimal at unheated temperatures (e.g., 32° C. or skin temperature), but diffusion can be increased significantly when heated. For example, by incorporating excipients and/or adhesives with melting or glass transition temperatures above skin temperature (e.g., 32° C.) into the transdermal membrane **10**, the membrane **10** matrix can allow diffusion of the drug from the transdermal membrane **10** to the skin upon heating. In one embodiment, small molecules or polymeric components with appropriate melting point or glass transition temperatures can be incorporated into the transdermal membrane **10**. This is in contrast to common transdermal matrix systems, which typically deliver active agent as soon as applied to the subject according to their active agent concentration driven controlled-release kinetics.

(25) FIG. 2 shows another transdermal delivery system **120** with temperature control element **3** and transdermal membrane **10** containing active agent **2** for delivery to a subject. Transdermal delivery system **120** is similar to transdermal delivery system **20**, but has two interconnectable parts **130**, **140** and a band **108**. The first part **130** includes transdermal membrane **10**. The second part **140** includes the band **108** and housing **17** (which includes temperature control element **3**, power source **4**, flexible circuit board **5**, microcontroller **6**, and communication chip **7**). The temperature control element **3** is configured to actively heat and/or actively cool a portion of the transdermal delivery device (e.g., transdermal membrane **10**) as described herein. In some embodiments, the first part **130** can be disposable and the second part **140** can be reusable.

(26) As shown in FIG. 2, the band **108** can be connected to housing **17** and is configured to hold housing **17** close to a subject. Band **108** may be configured to wrap around a subject's body, such as an arm or leg to hold housing **17** close to the subject. In some embodiments, band **108** may hold both first part **130** and second part **140** against a subject's body. First part **130** may additionally include an adhesive (as described above) with respect to FIG. 1, which may hold transdermal membrane **10** against a subject's body. In some embodiments, first part **130** is separable from second part **140**. For example, first part **130** and second part **140** may connect, such as through mating magnets on each part or mating snap-fit parts. In some other embodiments, first part **130** and second part **140** may be integral, e.g., not separable. In some embodiments, housing **17** may not contain all of the elements of the temperature control element **3**, power source **4**, flexible circuit board **5**, microcontroller **6**, and communication chip **7**. For example, housing **17** may contain temperature control element **3** while communication chip **7** may be absent or may be contained in first part **130**.

(27) FIG. 3 shows another transdermal delivery system **220** useful for controlling active agent delivery to a subject. Delivery system **220** is similar to system **120** (and includes two parts **230**, **240**) except that the first part **230**, which can be disposable, contains reactive material **222** to hold or otherwise prevent the active agent **2** from flowing across the transdermal membrane, and the second part **240**, which can be reusable, is configured to produce stimulus **223** to act on the reactive material **222** and control the active agent **2** flow. The delivery system **220** thus includes the transdermal membrane **10** with reactive material **222** and active agent **2** and temperature control element **3** configured to provide a stimulus **223** to reactive material **222** to control active agent **2** flow. The reactive material **222** can be a plurality of small molecules, a solvent, or a polymeric material that is responsive to the stimulus **223** from the temperature control element **3**. The reactive material **222** may include, for example, a polymer with a desired glass transition profile, a solvent, gold covered nanoparticles responsive to heat, and/or magnetic (e.g., iron) nanoparticles responsive to electromagnetic radiation. For example, reactive material **222** may be a polymer having a glass transition temperature ($T_{sub.g}$) configured to transition from a first form to a second form at a desired temperature. In the first form, the polymer may have a hard, solid, and/or glassy state that prevents or minimizes active agent **2** flow and in the second form, the polymer may have a softer, more rubbery, and/or more viscous state that allows active agent flow. The stimulus **223** may be heat or other energy from the temperature control element **3** configured to transition the polymer from the first form to the second form. The reactive material **222** may be in a relatively hard, solid and/or glassy state at lower temperatures, such as at room temperature and/or skin temperature (e.g., at less than 32° C., less than 33° C., less than 34° C., or less than 35° C.). The reactive material **222** may be in a softer, more rubbery, and/or more viscous state at higher temperatures, such as above room temperature and skin temperature, (e.g., at least 34° C., at least 35° C., at least 36° C., at least 37° C., at least 38° C., at least 39° C., or at least 40° C.). The reactive material **222** may prevent substantial diffusion of the active agent **2** when the reactive material **222** is in the relatively hard, solid and/or glassy state at a lower temperature. The reactive material **222** may release or otherwise allow the active agent **2** to flow to the skin **1** of a subject when the reactive material **222** is in the softer, more rubbery, and/or more viscous state at a higher temperature. For example, the reactive material **222** may include an epoxy, a polyethylene, a polymethacrylate, a polypropylene, a polypropylene glycol, a polyvinylacetate, a polystyrene, a polytetrafluoroethylene, a poly(bisphenol A carbonate), a poly(ethylene terephthalate), a polylactic acid (PLA), a polyglycolic acid (PGA), and/or a polyurethane. The reactive material **222** may be tuned to have a desired glass

transition temperature useful for holding and releasing a particular active agent in the transdermal system such as by including or excluding bulky, inflexible side groups, having relatively shorter or longer side chains, having a greater or lesser degree of crosslinking, or including or excluding a plasticizer such as nitrobenzene, B-naphthyl salicylate, carbon disulphide, glycerine, propylene glycol, triethyl citrate, triacetine, polyethylene glycol, having greater or less hydrophobicity. In some examples, the reactive material **222** may include a polymeric hydrogel.

(28) As indicated above, the delivery system **220** can include temperature control element **3** configured to deliver the stimulus **223** to reactive material **222** to control flow of active agent **2**. The stimulus **223** from temperature control element **3** may be, for example, kinetic energy, a magnetic force, an electrical pulse, infrared radiation, or electromagnetic radiation. Temperature control element **3** may include chemicals (e.g., calcium chloride, iron particles, supersaturated sodium acetate), an electromagnet, an inductive coil, an infrared light source, phase change materials, and/or a visible light source to provide the stimulus **223**.

(29) In some variations, the stimulus **223** may be configured to act on the reactive material **222** such that the reactive material **222** holds or otherwise prevents active agent **2** from flowing across the transdermal membrane **10** to a subject. Turning the stimulus **223** off can allow a pulse of active agent **2** to flow across the transdermal membrane **10** to a subject. The cycle may be repeated multiple times to provide pulsatile active agent delivery, as described elsewhere herein.

(30) In some embodiments, the transdermal system **220** may include two or more different types of reactive materials, and the transdermal system **220** may be configured to control pulsatile delivery of the active agent **2** or to control delivery of two different kinds of active agent **2**. For example, a first type of reactive material **222** may have a first glass transition temperature (T_g) and may release its payload of active agent **2** at a first (lower) temperature (e.g., 36°C .) while a second type of reactive material **222** may have a second glass transition temperature (T_g) component and may release its payload of active agent **2** at a second (higher) temperature (e.g., 39°C .). Active agent delivery may be controlled by first delivering a first stimulus **223** to a portion of the transdermal system so the first type of reactive material **222** releases its payload of active agent **2**, optionally turning the first stimulus **223** off, and delivering a second stimulus **223** to a portion of the transdermal system so the second type of reactive material **222** releases its payload of active agent **2**. Such first and second payloads may release the same kind of active agent **2** (e.g., in a pulsatile manner) or may release different types of active agents.

(31) Although the reactive material **222** may be in the transdermal membrane **10**, it can additionally or alternatively be in another part of the transdermal delivery system **220** and may be configured to control pulsatile delivery. For example, the reactive material **222** can be in an active agent reservoir. The active agent reservoir can be separate from the transdermal membrane **10** and may be fluidically connected to it. The active agent reservoir may be configured to hold active agent **2**. The stimulus **223** can act on the reactive material **222** in the active agent reservoir to release the active agent **2** from the active agent reservoir and into the transdermal patch. The stimulus **223** can allow release of active agent **2** from the active agent reservoir all at one time. Alternatively, the stimulus **223** can allow pulsatile release of active agent **2** from the active agent reservoir. An active agent reservoir may be in any of the delivery systems described herein, and may be in a first part or a second part of the delivery system.

(32) In some variations, the active agent **2** can be the reactive material **222** responsive to stimulus **223**, and there is can be no separate reactive material **222** that controls active agent flow through the transdermal membrane **10**. In some embodiments, stopping stimulus **223** stops or significantly slows releasing, heating or another mechanism otherwise allowing active agent to flow across transdermal membrane **10**. Significantly slowing the releasing, heating or other mechanism allowing active agent flow reduces the level of active agent flow relative to the “on” or stimulated level, such that even if some active agent is flowed across the transdermal membrane **10**, it does not have the “on” effect and may be sufficiently low to have no therapeutic or detectable effect on the subject. In some embodiments, a stimulus **223** from temperature control element **3** may act on reactive material **222** to actively stop reactive material **222** from releasing or flowing active agent across transdermal membrane

10. For example, temperature control element **3** may include a heat sink, an endothermic ventilation fan, an evaporative based cooling mechanism, a sublimation based cooling mechanism, cyclic refrigeration, thermoelectric refrigeration, or magnetic refrigeration and may be configured to cool reactive material **222** and prevent active agent flow to a subject. In some embodiments, the temperature control element **3** may be configured to deliver both an “on” stimulus such as heat to turn on active agent flow across the transdermal membrane **10** and an “off” stimulus such as refrigeration to prevent or decrease active agent flow across the transdermal membrane **10**.

(33) Any of the transdermal systems described herein may have a discreet or low profile. Such a system may be worn with minimal notice by the user (or others) or with no or minimal discomfort while sleeping. In some embodiments, the transdermal system may be less than 50 cm.^{sup.3} in volume, less than 25 cm.^{sup.3} in volume, (e.g., 5 cm×10 cm×0.5 cm), less than 20 cm.^{sup.3} in volume, less than 15 cm.^{sup.3} in volume or less than 10 cm.^{sup.3} in volume. In some embodiments, the transdermal system may have dimensions that are less than 5 cm on a first side, less than 10 cm on a second side, and less than 0.5 cm on a third side. In some embodiments, the transdermal system may weigh less than 60 grams, less than 30 grams, less than 20 grams, less than 10 grams or less than 5 grams.

(34) Any of the transdermal systems described herein may be very quiet (or silent) during use such as producing less than 20 dB, less than 10 dB, or less than 5 dB of sound during use.

(35) In some embodiments, the transdermal systems may be held to a subject, such as by using the band **108** or strap that wraps around part of a user's body. The adhesive may be configured to adhere the transdermal system against the user's skin only using the adhesive. In some embodiments, the system may be sufficiently small, lightweight or flat such that the adhesive can readily hold the system in place on a user without the use of the band **108** or strap. The transdermal systems may be configured to be placed in any desired location on a user's body (e.g., abdomen, arm, back, leg, scalp, upper arm) using adhesive **14** and/or band **108**.

(36) Also described herein are methods for transdermally delivering an active agent. Some methods include actively heating or cooling a portion of a transdermal active agent delivery device for a first amount of time. Some methods include the step of after the first amount of time, actively heating or cooling the portion of the transdermal active agent delivery device for a second amount of time. In some such methods, the actively heating or cooling steps provide pulsatile delivery of active agent from a transdermal membrane of the transdermal active agent delivery device to skin of a patient. In some methods, the actively heating or cooling steps comprises delivering pulsatile heat to the portion of the drug delivery device. In some methods, the actively heating or cooling steps comprises removing heat from the portion of the transdermal active agent delivery device. In some methods, at least one of the actively heating or cooling steps comprises delivering electromagnetic radiation to the portion of the drug delivery device. Some methods include repeating the actively heating or cooling steps at least once. Some methods include repeating the actively heating or cooling steps at least two times. Some methods include repeating the actively heating or cooling steps at least three times. In some methods, the portion of the transdermal delivery system is heated by at least 4° C. Some methods include actively heating skin of the subject adjacent the transdermal membrane by at least 3° C. In some methods, at least one of the actively heating or cooling steps is performed while the subject is sleeping.

(37) FIG. **4** shows results from using a transdermal delivery system as described herein with periodic heating and cooling. Experiments were carried out by applying passive matrix-type nicotine patches containing 21 mg of nicotine to human cadaver skin and then periodically heating and cooling the patches over a period of 20 hours. Controls with no heating and cooling were tested concurrently. The test patches were subject to 3 cycles of heating and cooling while the control patches were not. Patches were heated for 2 hours, cooled for 5 hours, heated for 2 hours, cooled for 4 hours, heated for 2 hours, and cooled. Nicotine delivery across the skin was measured and averaged for control patches (n=2) and test patches (n=4). Nicotine flow across the skin increased when heat was applied, and reached levels around 4.9 mg/hr, around 2.5 mg/hr, and around 1.8 mg/hr, respectively, during each of the three time periods when heat was applied. The level of nicotine delivery across the skin rapidly increased with heating and rapidly decreased with cooling as noted by the steep slope of increased nicotine flow after each heat application and steep decline of nicotine delivery after each cooling application. FIG. **4** shows

that pulsatile active agent (nicotine) delivery was achieved by alternate cycles of heating and cooling.

(38) Any of the systems or methods described herein, and in particular in a method for delivering an active agent to a subject, may include or be configured to include at least one of following or acceptable salts thereof, Acamprosate, Acetaminophen, Acetaminophen+Oxycodone, Aleve, Alevicyn SG, Alfentanil, Allopurinol, Almotriptan, Alprazolam, Amitriptyline, Amoxapine, Apomorphine, Aripiprazole, Armodafinil, Asenapinemaleate, Atomoxetine, Azelastine HCl, Baclofen, Benzbromarone, Benzydamine, Brexpiprazole, Budesonide, Bupivacaine, Buprenorphine, Buprenorphine+Naloxone, Bupropion, Bupropion Hydro bromide, Bupropion Hydrochloride, Buspirone, Cabergoline, Capsaicin, Carbamazepine, Carbidopa+Levodopa, Carisprodol, Celecoxib, Citalopram, Clobazam, Clonazepam, Clonidine, Clopidogrel, Colchicine, Cyclobenzaprine, Dalteparin sodium, Desvenlafaxine, Dexamfetamine, Dexmethylphenidate HCl, Diazepam, Diclofenac, Diclofenac Potassium, Disulfiram, Divalproex Sodium, Dolasetron Mesilate, Doxepin, Dronabinol, Droxidopa, Duloxetine, Eletriptan, Entacapone, Escitalopram oxalate, Eslicarbazepine Acetate, Esomeprazole/naproxen, Estradiol, Estrogen, Eszopiclone, Ethosuximide, Etodolac, Ezogabine, Febuxostat, Felbamate, Fenbufen, Fentanyl, Fentanyl Citrate, Fentanyl HCl Flunisolide, Fluorouracil, Fluoxetine, Fluticasone propionate, Fluvoxamine, Formoterol, Fosphenytoin, Frovatriptan, Gabapentin, Granisetron, Guanfacine, Hydrocodone Bitartrate, Hydrocodone+Acetaminophen, hydrocortisone, Hydromorphone HCl, Hydroxyzine, Hypericum Extract, Ibuprofen, Indometacin, Ketorolac, Lacosamide, Lamotrigine, Levetiracetam, Levomilnacipran, Levosalbutamol, Lidocaine, Lidocaine/Tetracaine, Lisdexamfetamine, Lithium Carbonate, Lorazepam, Lorcaserin Hydrochloride, Losartan, Loxapine, Meclizine, Meloxicam, Metaxalone, Methylphenidate, Methylphenidate Hydrochloride, Milnacipran, Mirtazapine, Modafinil, Morphine, Nabilone, Nadolol, Naltrexone, Naproxen, Naratriptan, Nedocromil, Nefazodone, Nicotine, Nicotine Sulfate, Nicotine salts, Nitroglycerin, Olanzapine, Ondansetron, Orlistat, Oxaprozin, Oxcarbazepine, Oxybutynin, Oxycodone, Oxycodone+Acetaminophen, Oxycodone Hydrochloride, Oxymorphone, Palonosetron, Pamidronate, Paroxetine, Paroxetine Mesylate, Perampanel, Phentermine+Topiramate, Phentermine Hydrochloride, Phentolamine Mesylate, Pramipexole, Prasugrel, Prazepam, Prednisone, Pregabalin, Promethazine, Propofol, Quetiapine, Quetiapine Fumarate, Ramelteon, Rasagiline, Rasagiline Mesylate, Remifentanyl, Risperidone, Rivastigmine, Rivastigmine Tartrate, Rizatriptan, Ropinirole, Ropivacaine, Rotigotine, Rufinamide, Salbutamol, Scopolamine, Selegiline, Sertraline, Sodium Oxybate, Strontium, Sufentanyl, Sumatriptan, Sumatriptan Succinate, Suvorexant, Tapentadol, Tasimelteon, Temazepam, Testosterone, Tetracaine+Lidocaine, Theophylline, Tiagabine, Tiotropium, TirofibanHCl, Tolcapone, Topiramate, Tramadol, Tramadol+Acetaminophen, Trazodone, Triazolam, Trimipramine Maleate, Valproate Semi sodium, Valproate Sodium, Venlafaxine, Vigabatrin, Vilazodone, Vortioxetine, Zaleplon, Zileuton, Ziprasidone, Zolmitriptan, Zolpidem, Zolpidem Tartrate, Norethisterone Acetate (NETA), Enapril, Ethinyl Estradiol, Insulin, Memantine, Methamphetamine, Norelgestromine, Pergolide, Ramipril, Tegrin, Timolol, Tolterodine and Zonisamide.

(39) In some particular examples, the systems or methods described herein may be configured to aid in smoking cessation or to treat aspects of Parkinson's Disease. For example, in some particular examples, the systems or methods described herein may include nicotine, a nicotine analog, a nicotine antagonist, a nicotine agonist, benztropine (Cogentin), carbidopa, dopamine, a dopamine analog, a dopamine antagonist, a dopamine agonist, entacapone, levodopa (L-dopa), pramipexole (Mirapex), rasagiline (Azilect®), ropinirole (Requip), rotigotine (Neupro®), safinamide (Xadago), selegiline (Eldepryl®, Zelapar™), sinemet (both carbidopa and levodopa), and trihexyphenidyl (Artane®), and tolcapone (Tasmar).

(40) The active agent 2 may be in the form of a solution, a suspension, a gel, or a dispersion. In some embodiments, the transdermal delivery system contains the active agent 2. In some embodiments, a transdermal delivery system as described herein may be replenished with active agent from an outside source and in some embodiments, a transdermal delivery system as described herein may not contain an active agent and may be configured to receive added active agent. For example, an active agent may be added to the transdermal delivery system by injection of active agent into the transdermal delivery system (e.g., into the transdermal membrane) or by adding an active agent reservoir configured to flow

active agent to the delivery system.

(41) It should be understood that features described with respect to one embodiment may be substituted for or used in addition to features described with respect to another embodiment.

(42) The systems and methods described herein can further include any of the elements or steps described in U.S. Pat. Nos. 8,673,346, 9,555,277, 8,372,040, 10,105,487, 10,213,586, U.S. Publication No. 2018/0374381, PCT Publication No. WO2018/106723, PCT Publication No. WO2018/129304, PCT Publication No. WO2018/148746, PCT Publication No. WO2018/129363, U.S. Publication No. US2018/0110768, or U.S. Publication No. 2019/0054078, the entireties of which are incorporated by reference herein.

(43) When a feature or element is herein referred to as being “on” another feature or element, it can be directly on the other feature or element or intervening features and/or elements may also be present. In contrast, when a feature or element is referred to as being “directly on” another feature or element, there are no intervening features or elements present. It will also be understood that, when a feature or element is referred to as being “connected”, “attached” or “coupled” to another feature or element, it can be directly connected, attached or coupled to the other feature or element or intervening features or elements may be present. In contrast, when a feature or element is referred to as being “directly connected”, “directly attached” or “directly coupled” to another feature or element, there are no intervening features or elements present. Although described or shown with respect to one embodiment, the features and elements so described or shown can apply to other embodiments. It will also be appreciated by those of skill in the art that references to a structure or feature that is disposed “adjacent” another feature may have portions that overlap or underlie the adjacent feature.

(44) Terminology used herein is for the purpose of describing particular embodiments only and is not intended to be limiting of the invention. For example, as used herein, the singular forms “a”, “an” and “the” are intended to include the plural forms as well, unless the context clearly indicates otherwise. It will be further understood that the terms “comprises” and/or “comprising,” when used in this specification, specify the presence of stated features, steps, operations, elements, and/or components, but do not preclude the presence or addition of one or more other features, steps, operations, elements, components, and/or groups thereof. As used herein, the term “and/or” includes any and all combinations of one or more of the associated listed items and may be abbreviated as “/”.

(45) Spatially relative terms, such as “under”, “below”, “lower”, “over”, “upper” and the like, may be used herein for ease of description to describe one element or feature's relationship to another element(s) or feature(s) as illustrated in the figures. It will be understood that the spatially relative terms are intended to encompass different orientations of the device in use or operation in addition to the orientation depicted in the figures. For example, if a device in the figures is inverted, elements described as “under” or “beneath” other elements or features would then be oriented “over” the other elements or features. Thus, the exemplary term “under” can encompass both an orientation of over and under. The device may be otherwise oriented (rotated 90 degrees or at other orientations) and the spatially relative descriptors used herein interpreted accordingly. Similarly, the terms “upwardly”, “downwardly”, “vertical”, “horizontal” and the like are used herein for the purpose of explanation only unless specifically indicated otherwise.

(46) Although the terms “first” and “second” may be used herein to describe various features/elements (including steps), these features/elements should not be limited by these terms, unless the context indicates otherwise. These terms may be used to distinguish one feature/element from another feature/element. Thus, a first feature/element discussed below could be termed a second feature/element, and similarly, a second feature/element discussed below could be termed a first feature/element without departing from the teachings of the present invention.

(47) Throughout this specification and the claims which follow, unless the context requires otherwise, the word “comprise”, and variations such as “comprises” and “comprising” means various components can be co-jointly employed in the methods and articles (e.g., compositions and apparatuses including device and methods). For example, the term “comprising” will be understood to imply the inclusion of any stated elements or steps but not the exclusion of any other elements or steps.

(48) In general, any of the apparatuses and methods described herein should be understood to be

inclusive, but all or a sub-set of the components and/or steps may alternatively be exclusive, and may be expressed as “consisting of” or alternatively “consisting essentially of” the various components, steps, sub-components or sub-steps.

(49) As used herein in the specification and claims, including as used in the examples and unless otherwise expressly specified, all numbers may be read as if prefaced by the word “about” or “approximately,” even if the term does not expressly appear. The phrase “about” or “approximately” may be used when describing magnitude and/or position to indicate that the value and/or position described is within a reasonable expected range of values and/or positions. For example, a numeric value may have a value that is $\pm 0.1\%$ of the stated value (or range of values), $\pm 1\%$ of the stated value (or range of values), $\pm 2\%$ of the stated value (or range of values), $\pm 5\%$ of the stated value (or range of values), $\pm 10\%$ of the stated value (or range of values), etc. Any numerical values given herein should also be understood to include about or approximately that value, unless the context indicates otherwise. For example, if the value “10” is disclosed, then “about 10” is also disclosed. Any numerical range recited herein is intended to include all sub-ranges subsumed therein. It is also understood that when a value is disclosed that “less than or equal to” the value, “greater than or equal to the value” and possible ranges between values are also disclosed, as appropriately understood by the skilled artisan. For example, if the value “X” is disclosed the “less than or equal to X” as well as “greater than or equal to X” (e.g., where X is a numerical value) is also disclosed. It is also understood that the throughout the application, data is provided in a number of different formats, and that this data, represents endpoints and starting points, and ranges for any combination of the data points. For example, if a particular data point “10” and a particular data point “15” are disclosed, it is understood that greater than, greater than or equal to, less than, less than or equal to, and equal to 10 and 15 are considered disclosed as well as between 10 and 15. It is also understood that each unit between two particular units are also disclosed. For example, if 10 and 15 are disclosed, then 11, 12, 13, and 14 are also disclosed.

(50) Although various illustrative embodiments are described above, any of a number of changes may be made to various embodiments without departing from the scope of the invention as described by the claims. For example, the order in which various described method steps are performed may often be changed in alternative embodiments, and in other alternative embodiments one or more method steps may be skipped altogether. Optional features of various device and system embodiments may be included in some embodiments and not in others. Therefore, the foregoing description is provided primarily for exemplary purposes and should not be interpreted to limit the scope of the invention as it is set forth in the claims.

(51) The examples and illustrations included herein show, by way of illustration and not of limitation, specific embodiments in which the subject matter may be practiced. As mentioned, other embodiments may be utilized and derived there from, such that structural and logical substitutions and changes may be made without departing from the scope of this disclosure. Such embodiments of the inventive subject matter may be referred to herein individually or collectively by the term “invention” merely for convenience and without intending to voluntarily limit the scope of this application to any single invention or inventive concept, if more than one is, in fact, disclosed. Thus, although specific embodiments have been illustrated and described herein, any arrangement calculated to achieve the same purpose may be substituted for the specific embodiments shown. This disclosure is intended to cover any and all adaptations or variations of various embodiments. Combinations of the above embodiments, and other embodiments not specifically described herein, will be apparent to those of skill in the art upon reviewing the above description.

Claims

1. A transdermal delivery system comprising: a transdermal membrane; a first active agent contained within a reservoir separate from and fluidically connected to the transdermal membrane, the transdermal membrane configured to control, in response to temperature, a flow of the first active agent from the reservoir to skin of a subject; and a temperature control element configured to cyclically heat

to a first temperature and/or cool a portion of the transdermal delivery system so as to provide pulsatile delivery of the first active agent through the transdermal membrane, wherein the temperature control element is further configured to cyclically heat to a second temperature, different from the first temperature, the portion of the transdermal delivery system so as to provide pulsatile delivery of a second active agent through the transdermal membrane.

2. The transdermal delivery system of claim 1, wherein the temperature control element comprises an electromagnetic energy source.

3. The transdermal delivery system of claim 1, wherein the temperature control element comprises a resistive element.

4. The transdermal delivery system of claim 1, wherein the temperature control element comprises an inductive coil or an electromagnet.

5. The transdermal delivery system of claim 1, wherein the temperature control element comprises a coolant or heat sink.

6. The transdermal delivery system of claim 1, wherein the portion of the transdermal membrane comprises a polymer configured to be heated or cooled to thereby change active agent flow.

7. The transdermal delivery system of claim 1, wherein the portion of the transdermal membrane comprises a glass transition polymer configured to be heated or cooled to thereby change active agent flow.

8. The transdermal delivery system of claim 1, wherein the portion of the transdermal membrane comprises a magnetic nanoparticle configured to be heated or cooled to thereby change active agent flow.

9. The transdermal delivery system of claim 1, wherein the first active agent comprises nicotine or a nicotine agonist.

10. The transdermal delivery system of claim 1, wherein the first active agent comprises a Parkinson's disease treatment.

11. The transdermal delivery system of claim 1, wherein the first active agent comprises at least one of benztropine, carbidopa, dopamine, a dopamine analog, a dopamine antagonist, a dopamine agonist, entacapone, levodopa (L-dopa), pramipexole, rasagiline, ropinirole, rotigotine, safinamide, selegiline, both carbidopa and levodopa, trihexyphenidyl and tolcapone.

12. The transdermal delivery system of claim 1, further comprising an adhesive in the transdermal membrane configured to adhere the transdermal membrane to the subject.

13. The transdermal delivery system of claim 1, further comprising an adhesive in the transdermal membrane, wherein the adhesive contains the first active agent.

14. The transdermal delivery system of claim 1, further comprising a temperature sensor configured to measure the temperature of at least one of the temperature control element, the portion of the transdermal delivery system, or the transdermal membrane.

15. The transdermal delivery system of claim 1, further comprising a power source configured to provide power to the temperature control element.

16. The transdermal delivery system of claim 1, further comprising a circuit board.

17. The transdermal delivery system of claim 1, further comprising a microcontroller configured to control delivery of a stimulus from the temperature control element.

18. The transdermal delivery system of claim 1, further comprising a communication element configured to receive or transmit data.

19. The transdermal delivery system of claim 18, wherein the communication element comprises Bluetooth or WiFi.

20. A method for transdermally delivering an active agent, comprising: cyclically heating to a first temperature and cooling a portion of a transdermal delivery system for a first amount of time; and after the first amount of time, cyclically heating to a second temperature and cooling the portion of the transdermal delivery system for a second amount of time, wherein the cyclic heating or cooling steps provide a pulsatile delivery of a first active agent from a reservoir based on the first temperature and a pulsatile deliver of a second active agent based on the second temperature, through a transdermal membrane of the transdermal delivery system to skin of a patient, wherein the transdermal membrane

controls a flow of the first active agent and the second active agent in response to temperature.

21. The method of claim 20, wherein the cyclically heating or cooling steps comprise delivering heat to the portion of the transdermal delivery system.

22. The method of claim 20, wherein the cyclically heating or cooling steps comprise removing heat from the portion of the transdermal delivery system.

23. The method of claim 20, wherein at least one of the cyclically heating or cooling steps comprises delivering electromagnetic radiation to the portion of the transdermal delivery system.

24. The method of claim 20, further comprising repeating the cyclic heating or cooling steps at least once.

25. The method of claim 20, further comprising repeating the cyclic heating or cooling steps at least twice.

26. The method of claim 20, wherein the portion of the transdermal delivery system is increased in temperature by at least 4° C. during the cyclic heating and cooling steps.

27. The method of claim 20, further comprising actively heating skin of the subject adjacent the transdermal membrane by at least 3° C. during the cyclic heating or cooling steps.

28. The method of claim 20, wherein at least one of the actively heating or cooling steps is performed while the subject is sleeping.

29. A transdermal delivery system comprising: a transdermal membrane; a first active agent contained within a reservoir separate from and fluidically connected to the transdermal membrane, the transdermal membrane configured to control, in response to temperature, a flow of the active agent from the reservoir to skin of a subject; and a temperature control element configured to provide a first stimulus to a portion of the transdermal delivery system so as to provide pulsatile delivery of the first active agent through the transdermal membrane to the skin of the subject and provide a second stimulus to the portion of the transdermal delivery system so as to provide pulsatile delivery of a second active agent through the transdermal membrane, wherein the first stimulus is a first temperature and the second stimulus is a second temperature that is different than the first temperature.

30. The transdermal delivery system of claim 29, further comprising a reactive material configured to prevent the active agent from flowing across the transdermal membrane until the stimulus is applied.

31. The transdermal delivery system of claim 30, wherein the reactive material comprises an epoxy, a polyethylene, a polymethacrylate, a polypropylene, a polypropylene glycol, a polyvinylacetate, a polystyrene, a polytetrafluoroethylene, a poly(bisphenol A carbonate), a poly(ethylene terephthalate), a polylactic acid (PLA), a polyglycolic acid (PGA), or a polyurethane.

32. The transdermal delivery system of claim 30, wherein the reactive material comprises a polymer, a hydrogel, a solvent, gold covered nanoparticles, or magnetic nanoparticles.
